

SWHS 365: GROUP COUNSELING (BAKER- NAUMAN)



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This text was compiled on 06/16/2026

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CHAPTER OVERVIEW

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1.1: The Psychology of Groups

Learning Objectives

16.1 Introduction to the Psychology of Groups

This module assumes that a thorough understanding of people requires a thorough understanding of groups. Each of us is an autonomous individual seeking our own objectives, yet we are also members of groups—groups that constrain us, guide us, and sustain us. Just as each of us influences the group and the people in the group, so, too, do groups change each one of us. Joining groups satisfies our need to belong, gain information and understanding through social comparison, define our sense of self and social identity, and achieve goals that might elude us if we worked alone. Groups are also practically significant, for much of the world's work is done by groups rather than by individuals. Success sometimes eludes our groups, but when group members learn to work together as a cohesive team their success becomes more certain. People also turn to groups when important decisions must be made, and this choice is justified as long as groups avoid such problems as group polarization and groupthink.



How many groups are you a part of on a daily basis? Whether it's family, class, work, social, sports, church or other areas, we typically spend a good deal of our time and attention each day interacting with others in groups. [Image: CC0 Public Domain, goo.gl/m25gce]

Psychologists study groups because nearly all human activities—working, learning, worshiping, relaxing, playing, and even sleeping—occur in groups. The lone individual who is cut off from all groups is a rarity. Most of us live out our lives in groups, and these groups have a profound impact on our thoughts, feelings, and actions. Many psychologists focus their attention on single individuals, but social psychologists expand their analysis to include groups, organizations, communities, and even cultures.

This module examines the psychology of groups and group membership. It begins with a basic question: What is the psychological significance of groups? People are, undeniably, more often in groups rather than alone. What accounts for this marked gregariousness and what does it say about our psychological makeup? The module then reviews some of the key findings from studies of groups. Researchers have asked many questions about people and groups: Do people work as hard as they can when they are in groups? Are groups more cautious than individuals? Do groups make wiser decisions than single individuals? In many cases the answers are not what common sense and folk wisdom might suggest.

16.2 The Psychological Significance of Groups

Many people loudly proclaim their autonomy and independence. Like Ralph Waldo Emerson, they avow, “I must be myself. I will not hide my tastes or aversions . . . I will seek my own” (1903/2004, p. 127). Even though people are capable of living separate and apart from others, they join with others because groups meet their psychological and social needs.

The Need to Belong



The need to belong is a strong psychological motivation. [Image: CC0 Public Domain, goo.gl/m25gce]

Across individuals, societies, and even eras, humans consistently seek inclusion over exclusion, membership over isolation, and acceptance over rejection. As Roy Baumeister and Mark Leary conclude, humans have a *need to belong*: “a pervasive drive to form and maintain at least a minimum quantity of lasting, positive, and impactful interpersonal relationships” (1995, p. 497). And most of us satisfy this need by joining groups. When surveyed, 87.3% of Americans reported that they lived with other people, including family members, partners, and roommates (Davis & Smith, 2007). The majority, ranging from 50% to 80%, reported regularly doing things in groups, such as attending a sports event together, visiting one another for the evening, sharing a meal together, or going out as a group to see a movie (Putnam, 2000).

People respond negatively when their need to belong is unfulfilled. For example, college students often feel homesick and lonely when they first start college, but not if they belong to a cohesive, socially satisfying group (Buote et al., 2007). People who are accepted members of a group tend to feel happier and more satisfied. But should they be rejected by a group, they feel unhappy, helpless, and depressed. Studies of *ostracism*—the deliberate exclusion from groups—indicate this experience is highly stressful and can lead to depression, confused thinking, and even aggression (Williams, 2007). When researchers used a functional magnetic resonance imaging scanner to track neural responses to exclusion, they found that people who were left out of a group activity displayed heightened cortical activity in two specific areas of the brain—the dorsal anterior cingulate cortex and the anterior insula. These areas of the brain are associated with the experience of physical pain sensations (Eisenberger, Lieberman, & Williams, 2003). It hurts, quite literally, to be left out of a group.

Affiliation in Groups

Groups not only satisfy the need to belong, they also provide members with information, assistance, and social support. Leon Festinger’s theory of *social comparison* (1950, 1954) suggested that in many cases people join with others to evaluate the accuracy of their personal beliefs and attitudes. Stanley Schachter (1959) explored this process by putting individuals in ambiguous, stressful situations and asking them if they wished to wait alone or with others. He found that people *affiliate* in such situations—they seek the company of others.

Although any kind of companionship is appreciated, we prefer those who provide us with reassurance and support as well as accurate information. In some cases, we also prefer to join with others who are even worse off than we are. Imagine, for example, how you would respond when the teacher hands back the test and yours is marked 85%. Do you want to affiliate with a friend who got a 95% or a friend who got a 78%? To maintain a sense of self-worth, people seek out and compare themselves to the less fortunate. This process is known as *downward social comparison*.

Identity and Membership

Groups are not only founts of information during times of ambiguity, they also help us answer the existentially significant question, “Who am I?” Common sense tells us that our sense of self is our private definition of who we are, a kind of archival record of our experiences, qualities, and capabilities. Yet, the self also includes all those qualities that spring from memberships in groups. People are defined not only by their traits, preferences, interests, likes, and dislikes, but also by their friendships, social roles, family connections, and group memberships. The self is not just a “me,” but also a “we.”

Even demographic qualities such as sex or age can influence us if we categorize ourselves based on these qualities. *Social identity theory*, for example, assumes that we don’t just classify *other* people into such social categories as man, woman, Anglo, elderly, or college student, but we also categorize ourselves. Moreover, if we strongly identify with these categories, then we will ascribe the characteristics of the typical member of these groups to

ourselves, and so stereotype ourselves. If, for example, we believe that college students are intellectual, then we will assume we, too, are intellectual if we identify with that group (Hogg, 2001).

Groups also provide a variety of means for maintaining and enhancing a sense of self-worth, as our assessment of the quality of groups we belong to influences our [collective self-esteem](#) (Crocker & Luhtanen, 1990). If our self-esteem is shaken by a personal setback, we can focus on our group's success and prestige. In addition, by comparing our group to other groups, we frequently discover that we are members of the better group, and so can take pride in our superiority. By denigrating other groups, we elevate both our personal and our collective self-esteem (Crocker & Major, 1989).

Mark Leary's [sociometer model](#) goes so far as to suggest that "self-esteem is part of a sociometer that monitors peoples' relational value in other people's eyes" (2007, p. 328). He maintains self-esteem is not just an index of one's sense of personal value, but also an indicator of acceptance into groups. Like a gauge that indicates how much fuel is left in the tank, a dip in self-esteem indicates exclusion from our group is likely. Disquieting feelings of self-worth, then, prompt us to search for and correct characteristics and qualities that put us at risk of social exclusion. Self-esteem is not just high self-regard, but the self-approbation that we feel when included in groups (Leary & Baumeister, 2000).

Evolutionary Advantages of Group Living

Groups may be humans' most useful invention, for they provide us with the means to reach goals that would elude us if we remained alone. Individuals in groups can secure advantages and avoid disadvantages that would plague the lone individuals. In his theory of social integration, Moreland concludes that groups tend to form whenever "people become dependent on one another for the satisfaction of their needs" (1987, p. 104). The advantages of group life may be so great that humans are biologically prepared to seek membership and avoid isolation. From an evolutionary psychology perspective, because groups have increased humans' overall fitness for countless generations, individuals who carried genes that promoted solitude-seeking were less likely to survive and procreate compared to those with genes that prompted them to join groups (Darwin, 1859/1963). This process of natural selection culminated in the creation of a modern human who seeks out membership in groups instinctively, for most of us are descendants of "joiners" rather than "loners."

16.3 Motivation and Performance

Groups usually exist for a reason. In groups, we solve problems, create products, create standards, communicate knowledge, have fun, perform arts, create institutions, and even ensure our safety from attacks by other groups. But do groups always outperform individuals?

Social Facilitation in Groups

Do people perform more effectively when alone or when part of a group? Norman Triplett (1898) examined this issue in one of the first empirical studies in psychology. While watching bicycle races, Triplett noticed that cyclists were faster when they competed against other racers than when they raced alone against the clock. To determine if the presence of others leads to the psychological stimulation that enhances performance, he arranged for 40 children to play a game that involved turning a small reel as quickly as possible (see Figure 1). When he measured how quickly they turned the reel, he confirmed that children performed slightly better when they played the game in pairs compared to when they played alone (see Stroebe, 2012; Strube, 2005).

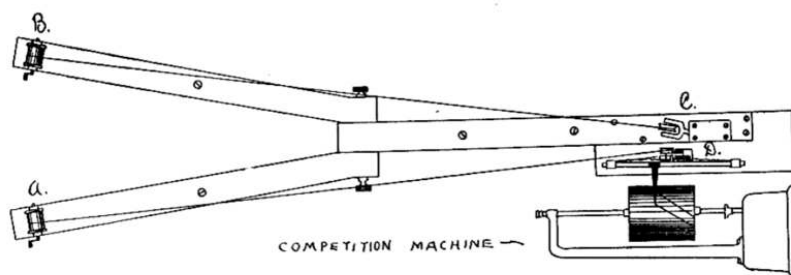


Figure 1: The "competition machine" Triplett used to study the impact of competition on performance. Triplett's study was one of the first laboratory studies conducted in the field of social psychology. Triplett, N. (1898)

Triplett succeeded in sparking interest in a phenomenon now known as [social facilitation](#): the enhancement of an individual's performance when that person works in the presence of other people. However, it remained for Robert Zajonc (1965) to specify when social facilitation does and does not occur. After reviewing prior research, Zajonc noted that the facilitating effects of an audience usually only occur when the task requires the person to perform dominant responses, i.e., ones that are well-learned or based on instinctive behaviors. If the task requires nondominant responses, i.e., novel, complicated, or untried behaviors that the organism has never performed before or has performed only infrequently, then the presence of others inhibits performance. Hence, students write poorer quality essays on complex philosophical questions when they labor in a group rather than alone (Allport, 1924), but they make fewer mistakes in solving simple, low-level multiplication problems with an audience or a coactor than when they work in isolation (Dashiell, 1930).

Social facilitation, then, depends on the task: other people facilitate performance when the task is so simple that it requires only dominant responses, but others interfere when the task requires nondominant responses. However, a number of psychological processes combine to influence when social facilitation, not social interference, occurs. Studies of the challenge-threat response and brain imaging, for example, confirm that we respond physiologically and neurologically to the presence of others (Blascovich, Mendes, Hunter, & Salomon, 1999). Other people also can trigger *evaluation apprehension*, particularly when we feel that our individual performance will be known to others, and those others might judge it negatively (Bond, Atoum, & VanLeeuwen, 1996). The presence of other people can also cause perturbations in our capacity to concentrate on and process information

(Harkins, 2006). Distractions due to the presence of other people have been shown to improve performance on certain tasks, such as the *Stroop task*, but undermine performance on more cognitively demanding tasks (Huguet, Galvaing, Monteil, & Dumas, 1999).

Social Loafing

Groups usually outperform individuals. A single student, working alone on a paper, will get less done in an hour than will four students working on a group project. One person playing a tug-of-war game against a group will lose. A crew of movers can pack up and transport your household belongings faster than you can by yourself. As the saying goes, “Many hands make light the work” (Littlepage, 1991; Steiner, 1972).

Groups, though, tend to be underachievers. Studies of social facilitation confirmed the positive motivational benefits of working with other people on well-practiced tasks in which each member’s contribution to the collective enterprise can be identified and evaluated. But what happens when tasks require a truly collective effort? First, when people work together they must coordinate their individual activities and contributions to reach the maximum level of efficiency—but they rarely do (Diehl & Stroebe, 1987). Three people in a tug-of-war competition, for example, invariably pull and pause at slightly different times, so their efforts are uncoordinated. The result is *coordination loss*: the three-person group is stronger than a single person, but not three times as strong. Second, people just don’t exert as much effort when working on a collective endeavor, nor do they expend as much cognitive effort trying to solve problems, as they do when working alone. They display *social loafing* (Latané, 1981).

Bibb Latané, Kip Williams, and Stephen Harkins (1979) examined both coordination losses and social loafing by arranging for students to cheer or clap either alone or in groups of varying sizes. The students cheered alone or in 2- or 6-person groups, or they were lead to believe they were in 2- or 6-person groups (those in the “pseudo-groups” wore blindfolds and headsets that played masking sound). As Figure 2 indicates, groups generated more noise than solitary subjects, but the productivity dropped as the groups became larger in size. In dyads, each subject worked at only 66% of capacity, and in 6-person groups at 36%. Productivity also dropped when subjects merely believed they were in groups. If subjects thought that one other person was shouting with them, they shouted 82% as intensely, and if they thought five other people were shouting, they reached only 74% of their capacity. These losses in productivity were not due to coordination problems; this decline in production could be attributed only to a reduction in effort—to social loafing (Latané et al., 1979, Experiment 2).

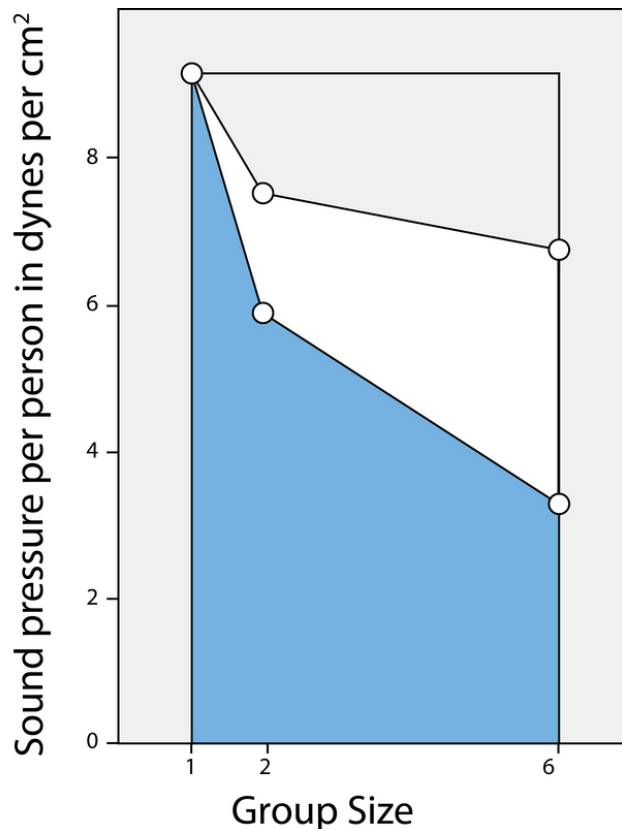


Figure 2: Sound pressure per person as a function of group or pseudo group size. Latane, B. (1981)

Teamwork



Social loafing can be a problem. One way to overcome it is by recognizing that each group member has an important part to play in the success of the group. [Image: Marc Dalmulder, <https://goo.gl/Xa5aiE>, CC BY 2.0, goo.gl/BRvSA7]

Social loafing is no rare phenomenon. When sales personnel work in groups with shared goals, they tend to “take it easy” if another salesperson is nearby who can do their work (George, 1992). People who are trying to generate new, creative ideas in group brainstorming sessions usually put in less effort and are thus less productive than people who are generating new ideas individually (Paulus & Brown, 2007). Students assigned group projects often complain of inequity in the quality and quantity of each member’s contributions: Some people just don’t work as much as they should to help the group reach its learning goals (Neu, 2012). People carrying out all sorts of physical and mental tasks expend less effort when working in groups, and the larger the group, the more they loaf (Karau & Williams, 1993).

Groups can, however, overcome this impediment to performance through teamwork. A group may include many talented individuals, but they must learn how to pool their individual abilities and energies to maximize the team’s performance. Team goals must be set, work patterns structured, and a sense of group identity developed. Individual members must learn how to coordinate their actions, and any strains and stresses in interpersonal relations need to be identified and resolved (Salas, Rosen, Burke, & Goodwin, 2009).

Researchers have identified two key ingredients to effective teamwork: a shared mental representation of the task and group unity. Teams improve their performance over time as they develop a shared understanding of the team and the tasks they are attempting. Some semblance of this shared mental model is present nearly from its inception, but as the team practices, differences among the members in terms of their understanding of their situation and their team diminish as a consensus becomes implicitly accepted (Tindale, Stawiski, & Jacobs, 2008).

Effective teams are also, in most cases, cohesive groups (Dion, 2000). Group cohesion is the integrity, solidarity, social integration, or unity of a group. In most cases, members of cohesive groups like each other and the group and they also are united in their pursuit of collective, group-level goals. Members tend to enjoy their groups more when they are cohesive, and cohesive groups usually outperform ones that lack cohesion.

This cohesion-performance relationship, however, is a complex one. Meta-analytic studies suggest that cohesion improves teamwork among members, but that performance quality influences cohesion more than cohesion influences performance (Mullen & Copper, 1994; Mullen, Driskell, & Salas, 1998; see Figure 3). Cohesive groups also can be spectacularly unproductive if the group’s norms stress low productivity rather than high productivity (Seashore, 1954).

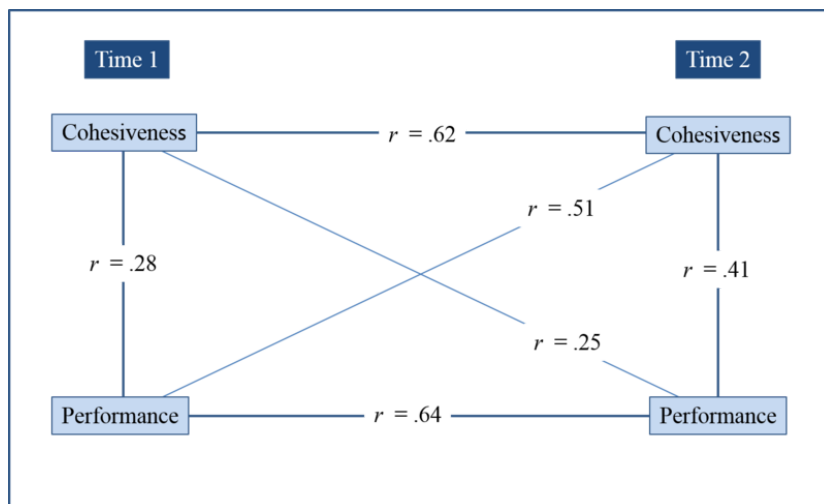


Figure 3: The relationship between group cohesion and performance over time. Groups that are cohesive do tend to perform well on tasks now (Time1) and in the future (Time 2). Notice, though, that the relationship between Performance at Time 1 and Cohesiveness at Time 2 is greater ($r=.51$) than the relationship between Cohesion at Time 1 and Performance at Time 2 ($r=.25$). These findings suggest that cohesion improves performance, but that a group that performs well is likely to also become more cohesive. Mullen, Driskell, & Salas (1998)

Group Development

In most cases, groups do not become smooth-functioning teams overnight. As Bruce Tuckman's (1965) theory of group development suggests, groups usually pass through several stages of development as they change from a newly formed group into an effective team. As noted in Focus Topic 1, in the *forming* phase, the members become oriented toward one another. In the *storming* phase, the group members find themselves in conflict, and some solution is sought to improve the group environment. In the *norming*, phase standards for behavior and roles develop that regulate behavior. In the *performing*, phase the group has reached a point where it can work as a unit to achieve desired goals, and the *adjourning* phase ends the sequence of development; the group disbands. Throughout these stages, groups tend to oscillate between the task-oriented issues and the relationship issues, with members sometimes working hard but at other times strengthening their interpersonal bonds (Tuckman & Jensen, 1977).

Focus Topic 1: Group Development Stages and Characteristics

Stage 1 – “Forming”. Members expose information about themselves in polite but tentative interactions. They explore the purposes of the group and gather information about each other's interests, skills, and personal tendencies.

Stage 2 – “Storming”. Disagreements about procedures and purposes surface, so criticism and conflict increase. Much of the conflict stems from challenges between members who are seeking to increase their status and control in the group.

Stage 3 – “Norming”. Once the group agrees on its goals, procedures, and leadership, norms, roles, and social relationships develop that increase the group's stability and cohesiveness.

Stage 4 – “Performing”. The group focuses its energies and attention on its goals, displaying higher rates of task-orientation, decision-making, and problem-solving.

Stage 5 – “Adjourning”. The group prepares to disband by completing its tasks, reduces levels of dependency among members, and dealing with any unresolved issues.

Sources based on Tuckman (1965) and Tuckman & Jensen (1977)

We also experience change as we pass through a group, for we don't become full-fledged members of a group in an instant. Instead, we gradually become a part of the group and remain in the group until we leave it. Richard Moreland and John Levine's (1982) model of group socialization describes this process, beginning with initial entry into the group and ending when the member exits it. For example, when you are thinking of joining a new group—a social club, a professional society, a fraternity or sorority, or a sports team—you investigate what the group has to offer, but the group also investigates you. During this investigation stage you are still an outsider: interested in joining the group, but not yet committed to it in any way. But once the group accepts you and you accept the group, socialization begins: you learn the group's norms and take on different responsibilities depending on your role. On a sports team, for example, you may initially hope to be a star who starts every game or plays a particular position, but the team may need something else from you. In time, though, the group will accept you as a full-fledged member and both sides in the process—you and the group itself—increase their commitment to one another. When that commitment wanes, however, your membership may come to an end as well.

16.4 Making Decisions in Groups

Groups are particularly useful when it comes to making a decision, for groups can draw on more resources than can a lone individual. A single individual may know a great deal about a problem and possible solutions, but his or her information is far surpassed by the combined knowledge of a group. Groups not only generate more ideas and possible solutions by discussing the problem, but they can also more objectively evaluate the options that they generate during discussion. Before accepting a solution, a group may require that a certain number of people favor it, or that it meets some other standard of acceptability. People generally feel that a group's decision will be superior to an individual's decision.

Groups, however, do not always make good decisions. Juries sometimes render verdicts that run counter to the evidence presented. Community groups take radical stances on issues before thinking through all the ramifications. Military strategists concoct plans that seem, in retrospect, ill-conceived and short-sighted. Why do groups sometimes make poor decisions?

Group Polarization

Let's say you are part of a group assigned to make a presentation. One of the group members suggests showing a short video that, although amusing, includes some provocative images. Even though initially you think the clip is inappropriate, you begin to change your mind as the group discusses the idea. The group decides, eventually, to throw caution to the wind and show the clip—and your instructor is horrified by your choice.

This hypothetical example is consistent with studies of groups making decisions that involve risk. Common sense notions suggest that groups exert a moderating, subduing effect on their members. However, when researchers looked at groups closely, they discovered many groups shift toward more extreme decisions rather than less extreme decisions after group interaction. Discussion, it turns out, doesn't moderate people's judgments after all. Instead, it leads to **group polarization**: judgments made after group discussion will be more extreme in the same direction as the average of individual judgments made prior to discussion (Myers & Lamm, 1976). If a majority of members feel that taking risks is more acceptable than exercising caution, then the group will become riskier after a discussion. For example, in France, where people generally like their government but dislike Americans, group discussion improved their attitude toward their government but exacerbated their negative opinions of Americans (Moscovici & Zavalloni, 1969). Similarly, prejudiced people who discussed racial issues with other prejudiced individuals became even more negative, but those who were relatively unprejudiced exhibited even more acceptance of diversity when in groups (Myers & Bishop, 1970).

Common Knowledge Effect

One of the advantages of making decisions in groups is the group's greater access to information. When seeking a solution to a problem, group members can put their ideas on the table and share their knowledge and judgments with each other through discussions. But all too often groups spend much of their discussion time examining common knowledge—information that two or more group members know in common—rather than unshared information. This **common knowledge effect** will result in a bad outcome if something known by only one or two group members is very important.

Researchers have studied this bias using the *hidden profile task*. On such tasks, information known to many of the group members suggests that one alternative, say Option A, is best. However, Option B is definitely the better choice, but all the facts that support Option B are only known to individual groups members—they are not common knowledge in the group. As a result, the group will likely spend most of its time reviewing the factors that favor Option A, and never discover any of its drawbacks. In consequence, groups often perform poorly when working on problems with nonobvious solutions that can only be identified by extensive information sharing (Stasser & Titus, 1987).

Groupthink



Groupthink helps us blend in and feel accepted and validated but it can also lead to problems. [Image: CC0 Public Domain, goo.gl/m25gce]

Groups sometimes make spectacularly bad decisions. In 1961, a special advisory committee to President John F. Kennedy planned and implemented a covert invasion of Cuba at the Bay of Pigs that ended in total disaster. In 1986, NASA carefully, and incorrectly, decided to launch the Challenger space shuttle in temperatures that were too cold.

Irving Janis (1982), intrigued by these kinds of blundering groups, carried out a number of case studies of such groups: the military experts that planned the defense of Pearl Harbor; Kennedy's Bay of Pigs planning group; the presidential team that escalated the war in Vietnam. Each group, he concluded, fell prey to a distorted style of thinking that rendered the group members incapable of making a rational decision. Janis labeled this syndrome **groupthink**: "a mode of thinking that people engage in when they are deeply involved in a cohesive in-group, when the members' strivings for unanimity override their motivation to realistically appraise alternative courses of action" (p. 9).

Janis identified both the telltale symptoms that signal the group is experiencing groupthink and the interpersonal factors that combine to cause groupthink. To Janis, groupthink is a disease that infects healthy groups, rendering them inefficient and unproductive. And like the physician who searches for symptoms that distinguish one disease from another, Janis identified a number of symptoms that should serve to warn members that they

may be falling prey to groupthink. These symptoms include overestimating the group's skills and wisdom, biased perceptions and evaluations of other groups and people who are outside of the group, strong conformity pressures within the group, and poor decision-making methods.

Janis also singled out four group-level factors that combine to cause groupthink: cohesion, isolation, biased leadership, and decisional stress.

- *Cohesion*: Groupthink only occurs in cohesive groups. Such groups have many advantages over groups that lack unity. People enjoy their membership much more in cohesive groups, they are less likely to abandon the group, and they work harder in pursuit of the group's goals. But extreme cohesiveness can be dangerous. When cohesiveness intensifies, members become more likely to accept the goals, decisions, and norms of the group without reservation. Conformity pressures also rise as members become reluctant to say or do anything that goes against the grain of the group, and the number of internal disagreements—necessary for good decision making—decreases.
- *Isolation*. Groupthink groups too often work behind closed doors, keeping out of the limelight. They isolate themselves from outsiders and refuse to modify their beliefs to bring them into line with society's beliefs. They avoid leaks by maintaining strict confidentiality and working only with people who are members of their group.
- *Biased leadership*. A biased leader who exerts too much authority over group members can increase conformity pressures and railroad decisions. In groupthink groups, the leader determines the agenda for each meeting, sets limits on discussion, and can even decide who will be heard.
- *Decisional stress*. Groupthink becomes more likely when the group is stressed, particularly by time pressures. When groups are stressed they minimize their discomfort by quickly choosing a plan of action with little argument or dissension. Then, through collective discussion, the group members can rationalize their choice by exaggerating the positive consequences, minimizing the possibility of negative outcomes, concentrating on minor details, and overlooking larger issues.

16.5 You and Your Groups



Even groups that like one another and work well together in most situations can be victims of groupthink or the common knowledge effect. But knowing that these pitfalls exist is the first step to overcoming them. [Image: CC0 Public Domain, goo.gl/m25gce]

Most of us belong to at least one group that must make decisions from time to time: a community group that needs to choose a fund-raising project; a union or employee group that must ratify a new contract; a family that must discuss your college plans; or the staff of a high school discussing ways to deal with the potential for violence during football games. Could these kinds of groups experience groupthink? Yes, they could, if the symptoms of groupthink discussed above are present, combined with other contributing causal factors, such as cohesiveness, isolation, biased leadership, and stress. To avoid polarization, the common knowledge effect, and groupthink, groups should strive to emphasize open inquiry of all sides of the issue while admitting the possibility of failure. The leaders of the group can also do much to limit groupthink by requiring full discussion of pros and cons, appointing devil's advocates, and breaking the group up into small discussion groups.

If these precautions are taken, your group has a much greater chance of making an informed, rational decision. Furthermore, although your group should review its goals, teamwork, and decision-making strategies, the human side of groups—the strong friendships and bonds that make group activity so enjoyable—shouldn't be overlooked. Groups have instrumental, practical value, but also emotional, psychological value. In groups, we find others who appreciate and value us. In groups, we gain the support we need in difficult times, but also have the opportunity to influence others. In groups, we find evidence of our self-worth, and secure ourselves from the threat of loneliness and despair. For most of us, groups are the secret source of well-being.

16.6 Outside Resources

Audio: This American Life. [Episode 109](#) deals with the motivation and excitement of joining with others at summer camp.

Audio: This American Life. [Episode 158](#) examines how people act when they are immersed in a large crowd.

Audio: This American Life. [Episode 61](#) deals with fiascos, many of which are perpetrated by groups.

Audio: This American Life. [Episode 74](#) examines how individuals act at conventions when they join with hundreds or thousands of other people who are similar in terms of their avocations or employment.

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Resources for the [Teaching of Social Psychology](#).

Social Psychology Network [Student Activities](#)

[Society for Social and Personality Psychology](#)

Tablante, C. B., & Fiske, S. T. (2015). [Teaching social class](#). *Teaching of Psychology*, 42, 184-190. doi:10.1177/0098628315573148 The article abstract can be viewed online. Visit your campus library to access the full-text version.

Video: Flash mobs illustrate the capacity of groups to organize quickly and complete complex tasks. One well-known example of a pseudo-flash mob is the rendition of "Do Re Mi" from the Sound of Music in the Central Station of Antwerp in 2009.



A YouTube element has been excluded from this version of the text. You can view it online here: pb.libretexts.org/humanbe/?p=89

Web: [Group Development](#) – This is a website developed by James Atherton that provides detailed information about group development, with application to the lifecycle of a typical college course.

Web: [Group Dynamics](#)– A general repository of links, short articles, and discussions examining groups and group processes, including such topics as crowd behavior, leadership, group structure, and influence.

Web: [Stanford Crowd Project](#) – This is a rich resource of information about all things related to crowds, with a particular emphasis on crowds and collective behavior in literature and the arts.

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SECTION OVERVIEW

1.2: Group Communication

Learning Objectives

After reading this chapter you should be able to:

- Define what constitutes a group and team.
- Understand cultural influences on groups.
- Explain how groups and teams form.
- Identify group roles and norms.
- Understand different styles of leadership in groups.
- Recognize style and options for decision making in groups.
- Explain the impact of technology on group communication.

1.2.1: Group Communication Overview

1.2.2: Defining Groups and Teams

1.2.3: The Importance of Studying Communication in Groups and Teams

1.2.4: Forming Groups

1.2.5: Groups Roles

1.2.6: Leadership In Groups

1.2.7: Decision Making In Groups

1.2.8: Group Communication Summary

1.2.9: Group Communication References

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1.2.1: Group Communication Overview

Have you ever had this happen to you in a college class? At the beginning of the semester your professor hands out the syllabus and explains that a group project is part of the course requirements. You, and others in the class, groan at the idea of this project because you have experienced the difficulties and frustrations of working in a group, especially when your grade depends on the work of others. Does this sound familiar? Why do you think so many students react negatively to these types of assignments? Group work can be fraught with complications. But, the reality is, many companies are promoting groups as the model working environment (Forbes).



Figure 1.2.1.1: “When I die, I would like the people I did group projects with to lower me in to my grave so they can let me down one last time.” – Jessica Clydesdale, frustrated student

Chances are that a class assignment is not your first and only experience with groups. Most likely, you have already spent, and will continue to spend, a great deal of your time working in groups. You may be involved with school athletics in which you are part of a specialized group called a team. You may be part of a work or professional group. Many of you participate in social, religious, and/or political groups. The family in which you were raised, regardless of the configuration, is also a group. No matter what the specific focus—sports, profession, politics, or family—all groups share some common features.

While group communication is growing in popularity and emphasis, both at the academic and corporate levels, it is not a new area of study. The emergence of group communication study came about in the mid 1950s, following World War II, and has been a focus of study ever since. Group communication is often closely aligned with interpersonal communication and organizational communication which is why we have placed it as a chapter in between these two areas of specialization. In your personal, civic, professional lives, you will engage in group communication. Let’s take a look at what constitutes a group or team.

Contributions and Affiliations

- Survey of Communication Study. **Authored by:** Scott T Paynton and Linda K Hahn. **Provided by:** Humboldt State University. **Located at:** https://en.wikibooks.org/wiki/Survey_of_Communication_Study. **License:** CC BY-SA: Attribution-ShareAlike
- Image of group working. **Authored by:** Spaynton. **Located at:** <https://commons.wikimedia.org/wiki/File:Groupwork490.png>. **License:** CC BY-SA: Attribution-ShareAlike

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1.2.2: Defining Groups and Teams

To understand group and team communication, we must first understand the definition of a group. Many people think that a group is simply a collection of people, but that is only part of it. If you walk out your front door and pull together the first ten people you see, do you have a group? No! According to Wilson and Hanna, **groups** are defined as, “a collection of three or more individuals who interact about some common problem or interdependent goal and can exert mutual influence over one another” (14). They goes on to say that the three key components of groups are, “size, goal orientation, and mutual influence” (14). Interpersonal communication is often thought about in terms of dyads, or pairs. Organizational communication might be thought of as a group that is larger than 12 people. While there are exceptions, for the most part, group size is often thought of in terms of 3-12 people.

Case In Point

Astronaut Jim Lovell’s words during the Apollo 13 lunar mission, “Houston, we have a problem,” launched a remarkable tale of effective teamwork and creative problem solving by NASA engineers working to try to save the lives of the jeopardized crew when two oxygen tanks exploded en route to the moon. Details of the dramatic and successful resolution to the problem became widely known in the motion picture Apollo 13, but it’s not just during dramatic moments when the importance of good teamwork is needed or recognized. In fact, some form of team-oriented work is encouraged in most, if not all, organizations today (Hughes, Jones). So if you feel this is unimportant to know, remember that group communication and teamwork skills are critical for your success later in life.

For example, Joseph Bonito, a communication professor at University of Arizona, allows no more than five people in a group to ensure that everyone’s opinions are reflected equally in a discussion. (Baughman and Everett-Haynes). According to Bonito, if there are too many people in a group it’s possible that some individuals will remain silent without anyone noticing. He suggests using smaller groups when equal participation is desirable. So, if the ten people you gathered outside of your front door were all neighbors working together as part of a “neighborhood watch” to create safety in the community, then you would indeed have a group.

For those of you who have participated on athletic teams you’ll notice that these definitions also apply to a team. While all of the qualities of groups hold true for teams, teams have additional qualities not necessarily present for all groups. We like to define a **team** as a *specialized group with a strong sense of belonging and commitment to each other that shapes an overall collective identity*. Members of a team each have their own part, or role, to fulfill in order to achieve the team’s greater goals. One member’s strengths can be other member’s weaknesses, so working in a team is beneficial when balancing individual input. While all members of an athletic team share some athletic ability and special appreciation for a particular sport, for example, members of a football team, for example, have highly specialized skills as indicated in the various positions on the team—quarterback, receiver, and running back. In addition to athletic teams, work and professional teams also share these qualities. Now that you know how to define groups and teams, let’s look at characteristics of groups and teams, as well as the different types of groups and teams.

Characteristics of Groups

- **Interdependence.** Groups cannot be defined simply as three or more people talking to each other or meeting together. Instead, a primary characteristic of groups is that members of a group are dependent on the others for the group to maintain its existence and achieve its goals. In essence, interdependence is the recognition by those in a group of their need for the others in the group (Lewin; Cragon, Wright& Kasch; Sherblom). Imagine playing in a basketball game as an individual against the five members of another team. Even if you’re considered the best basketball player in the world, it’s highly unlikely you could win a game against five other people. You must rely on four other teammates to make it a successful game.



- **Interaction.** It probably seems obvious to you that there must be interaction for groups to exist. However, what kind of interaction must exist? Since we all communicate every day, there must be something that distinguishes the interaction in groups from other forms of communication. Cragon, Wright and Kasch state that the primary defining characteristic of group interaction is that it is purposeful. They go on to break down purposeful interaction into four

types: *problem solving, role playing, team building, and trust building* (7). Without purposeful interaction a true group does not exist. Roles, norms, and relationships between members are created through interaction. If you're put into a group for a class assignment, for example, your first interaction probably centers around exchanging contact information, settings times to meet, and starting to focus on the task at hand. It's purposeful interaction in order to achieve a goal.

Group Communication Then

The first study that was published on group communication in the New School era of communication study was credited to Edwin Black in 1955. He studied the breakdowns in group interactions by looking at communication sequences in groups. However, it wasn't until the 1960s and 70s that a large number of studies in group communication began to appear. Between 1970 and 1978 114 articles were published on group communication and 89 more were published by 1990 (Stacks & Salwen 360). Study in group communication is still important over a decade later as more and more organizations focus on group work for achieving their goals.

- **Synergy.** One advantage of working in groups and teams is that they allow us to accomplish things we wouldn't be able to accomplish on our own. Remember back to our discussion of Systems Theory in Chapter 5. Systems Theory suggests that "The whole is greater than the sum of its parts." This is the very idea of synergy (Sherblom). In an orchestra or band, each person is there to perform in order to help the larger unit make music in a way that cannot be accomplished without each member working together.
- **Common Goals.** Having interaction and synergy would be relatively pointless in groups without a common goal. People who comprise groups are brought together for a reason or a purpose. While most members of a group have individual goals, a group is largely defined by the common goals of the group. Think of the example at the beginning of the chapter: Your common goal in a class group is to learn, complete an assignment, and earn a grade. While there may be differences regarding individual goals in the group (what final grade is acceptable for example), or how to achieve the common goals, the group is largely defined by the common goals it shares.
- **Shared Norms.** Because people come together for a specific purpose, they develop shared norms to help them achieve their goals. Even with a goal in place, random interaction does not define a group. Group interaction is generally guided by norms a group has established for acceptable behavior. Norms are essentially expectations of the group members, established by the group and can be conscious and formal, or unconscious and informal. A couple of examples of group norms include the expectation that all members show up at group meeting times, the expectation that all group members focus on the group instead of personal matters (for example, turning cell phones and other distractions off), and the expectation that group members finish their part of the work by the established due date. When members of the group violate group norms, other members of the group get frustrated and the group's overall goal may be affected.
- **Cohesiveness.** One way that members understand of the idea of communicating in groups and teams is when they experience a sense of cohesiveness with other members of the group. When we feel like we are part of something larger, we experience a sense of cohesion or wholeness, and may find a purpose that is bigger than our own individual desires and goals. It is the sense of connection and participation that characterizes the interaction in a group as different from the defined interaction among loosely connected individuals. If you've ever participated in a group that achieved its goal successfully, you are probably able to reflect back on your feelings of connections with the other members of that group.

You may be asking yourself, what about teams? We have focused primarily on groups, but it's critical to remember the importance of team communication characteristics as well as group communication characteristics. Check out [this article](#) that breaks down team characteristics and skills that ensure team success (we bet you'll find similarities to the group characteristics that we have just explained).

Types of Groups

Not all groups are the same or brought together for the same reasons. Brilhart and Galanes categorize groups "on the basis of the reason they were formed and the human needs they serve" (9). Let's take a look!

- **Primary Groups.** Primary groups are ones we form to help us realize our human needs like inclusion and affection. They are not generally formed to accomplish a task, but rather, to help us meet our fundamental needs as relational beings like acceptance, love, and affection. These groups are generally longer term than other groups and include family, roommates, and other relationships that meet as groups on a regular basis (Brilhart & Galanes). These special people in your life constitute primary groups because they offer love and support for the long run, and given this, primary groups are typically more meaningful than secondary groups.

- **Secondary Groups.** Unlike primary groups, we form secondary groups to accomplish work, perform a task, solve problems, and make decisions (Brilhart & Galanes; Sherblom; Cragan, Wright & Kasch). Larson and LaFasto state that secondary groups have “a specific performance objective or recognizable goal to be attained; and coordination of activity among the members of the team is required for attainment of the team goal or objective” (19).



- **Activity Groups.** Activity Groups are ones we form for the purpose of participating in activities. I’m sure your campus has many clubs that are organized for the sole purpose of doing activities. On our campus, for example, a popular club on our campus is the Surf Club, in which members meet for the purpose of scheduling surfing times, arranging rides, and choosing where to surf.
- **Personal Growth Groups.** We form Personal Growth Groups to obtain support and feedback from others, as well as to grow as a person. Personal Growth Groups may be thought of as therapy groups. An example that is probably familiar to you is Alcoholics Anonymous, where alcoholics can share their stories and struggles and get support from others affected by alcohol. There are many personal growth groups available for helping us develop as people through group interaction with others, such as book clubs, weight watchers, and spiritual groups.
- **Learning Groups.** Learning Groups focus mainly on obtaining new information and gaining knowledge. If you have ever been assigned to a group in a college class, it most likely was a learning group with the purpose of interacting in ways that can help those in the group learn new things about the course content.
- **Problem-Solving Groups.** These groups are created for the express purpose of solving a specific problem. The very nature of organizing people into this type of group is to get them to collectively figure out effective solutions to the problem they have before them. Committees are an excellent example of people who are brought together to solve problems.

After looking at the various types of groups, it’s probably easy for you to recognize just how much of your daily interaction occurs within the contexts of groups. The reality is, we spend a great deal of time in groups, and understanding the types of groups you’re in, as well as their purpose, goes a long way toward helping you function as a productive member.

Contributions and Affiliations

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1.2.3: The Importance of Studying Communication in Groups and Teams

One of the reasons communication scholars study groups and teams is because of the overwhelming amount of time we spend interacting in groups in professional contexts. More and more professional organizations are turning to groups and teams as an essential way of conducting business and getting things done. Even professions that are seemingly independent, such as being a college professor, are heavily laden with group work. The process of writing this book was a group effort as the authors and their students worked in groups to bring the book to you. Each of us had specific roles and tasks to perform.

Another vital area of group communication concerns the study of social change or social movement organizations. Groups such as People for the Ethical Treatment of Animals (PETA), the National Association for the Advancement of Colored People (NAACP), and the National Organization of Women (NOW) are all groups bound together by a shared social and political commitment—to promote the rights of nonhuman animals, African-Americans, and women respectively. While individuals can be committed to these ideas, the social, political, and legal rights afforded to groups like these would not have been possible through individual action alone. It was when groups of like-minded people came together with shared commitments and goals, pooling their skills and resources, that change occurred. Just about any Interest Group you can think of has a presence in Washington D.C. and spends money to maintain that presence. Visit [here for more information on Interest Groups](#).

The study of social movements reveals the importance of groups for accomplishing goals. Bowers, Ochs, Jensen and Schulz, in *The Rhetoric of Agitation and Control*, explain seven progressive and cumulative strategies through which movements progress as they move toward success. Three of the seven strategies focus explicitly on group communication—promulgation, solidification, and polarization. **Promulgation** refers to the “*a strategy where agitators publicly proclaim their goals and it includes tactics designed to win public support*” (23). Without a sufficient group the actions of individual protestors are likely to be dismissed. The strategy of **solidification** “*occurs mainly inside the agitating group*” and is “*primarily used to unite followers*” (29). The point is to unite group members and provide sufficient motivation and support. The communication that occurs through the collective action of singing songs or chanting slogans serves to unite group members. Because the success of social movements depends in part on the ability to attract a large number of followers, most employ the strategy of **polarization**, which is designed to *persuade neutral individuals or “fence sitters” to join a group* (40). The essence of this strategy is captured in the quote from Eldridge Cleaver, “You are either part of the problem or part of the solution.” Taken together these three strategies stress that the key to group success is the sustained effort of group members working together through communication.

Not only do Communication scholars focus on work and social movements, we are also interested in the role that one’s cultural identity and membership plays in our communicative choices, and how we interpret the communication of others. This focus sheds interesting insights when we examine membership and communication in groups and teams. One reason for this is that different cultures emphasize the role of individuals while other cultures emphasize the importance of the group. For example, **collectivist cultures** are ones that *place high value on group work because they understand that outcomes of our communication impact all members of the community and the community as a whole, not just the individuals in the group*. Conversely, **individualistic cultures** are ones that *place high value on the individual person above the needs of the group*. Thus, whether we view group work as favorable or unfavorable may stem from our cultural background. The U.S. is considered an individualistic culture in that we value the work and accomplishments of the individual through ideals such as being able to “pull yourself up by the bootstraps” and create success for yourself.

Case In Point

The Historic Number of Women in the 113th Congress

This Congress has 102 female members, including 20 female Senators, the most ever in the history of America. This was also an historic election for women of color. More than half of Asian Americans elected to the 113th Congress are women— including Mazie Hirono (D-HI), who is only the second woman of color and the only Asian American woman ever elected to the Senate. In the House, Grace Meng (D) will become New York’s first Asian American member of Congress, and Iraq War veteran Tulsi Gabbard (D-HI) the first American Samoan elected as a voting member of Congress. They will both join Tammy Duckworth (D-IL), a Thai-born, double amputee war hero. Among Latinas, Congresswoman-elect Michele Lujan Grisham became the first Hispanic woman elected representative from New Mexico.

[See the faces of the 113th Congress](#)

The Women's Caucus

The Congressional Caucus for Women's Issues (CCWI) is a Congressional Member Organization (CMO) registered with the House Administration Committee; its membership includes the women members of the House of Representatives. CCWI is not formally affiliated with Women's Policy, Inc.; however, the two organizations share similar goals. This relates to our collectivistic culture in organizational groups.

WPI is a nonprofit, nonpartisan organization whose sole focus is to help ensure that the most informed decisions on key women's issues are made by policymakers at the federal, state, and local levels. Audiences include elected officials, regulators, women's groups, labor groups, academia, the business community, the media, and the general public.

WPI achieves and shares its rare quality of insight into relevant issues by researching and producing the best available information in the form of compelling and unbiased legislative analyses, issue summaries, impact assessments, and educational briefings. This ensures that policy decisions affecting women and their families have the benefit of input from the most objective sources possible.

For more info on The Women's Caucus visit <http://www.womenspolicy.org/>

Power in Groups

Given the complexity of group interaction, it's short-sighted to try to understand group communication without looking at notions of power (think back to Critical Theories and Research Methods). **Power influences how we interpret the messages of others and determines the extent to which we feel we have the right to speak up and voice our concerns and opinions to others.** Take a moment to reflect on the different ways you think about power. What images come to mind for you when you think of power? Are there different kinds of power? Are some people inherently more powerful than others? Do you consider yourself to be a powerful person? We highlight three ways to understand power as it relates to group and team communication. The word "power" literally means "to be able" and has many implications.

If you associate power with control or dominance, this refers to the notion of power as **power-over**. According to Starhawk, "power-over enables one individual or group to make the decisions that affect others, and to enforce control" (9). Control can and does take many forms in society. Starhawk explains that,

This power is wielded from the workplace, in the schools, in the courts, in the doctor's office. It may rule with weapons that are physical or by controlling the resources we need to live: money, food, medical care; or by controlling more subtle resources: information, approval, love. We are so accustomed to power-over, so steeped in its language and its implicit threats, that we often become aware of its functioning only when we see its extreme manifestations. (9)

When we are in group situations and someone dominates the conversation, makes all of the decisions, or controls the resources of the group such as money or equipment, this is power-over.

Power-from-within refers to a more personal sense of strength or agency. Power-from-within manifests *itself when we can stand, walk, and speak "words that convey our needs and thoughts"* (Starhawk 10). In groups, this type of power "arises from our sense of connection, our bonding with other human beings, and with the environment" (10). As Heider explains in *The Tao of Leadership*, "Since all creation is a whole, separateness is an illusion. Like it or not, we are team players. Power comes through cooperation, independence through service, and a greater self through selflessness" (77). If you think about your role in groups, how have you influenced other group members? Your strategies indicate your sense of power-from-within.

Finally, groups manifest **power-with**, which is "*the power of a strong individual in a group of equals, the power not to command, but to suggest and be listened to, to begin something and see it happen*" (Starhawk 10). For this to be effective in a group or team,



at least two qualities must be present among members: 1) All group members must communicate respect and equality for one another, and 2) The leader must not abuse power-with and attempt to turn it into power-over. Have you ever been involved in a group where people did not treat each others as equals or with respect? How did you feel about the group? What was the outcome? Could you have done anything to change that dynamic?

Obviously, communication is the central activity of every group because it is how we organize and maintain groups. While we can all tell positive and negative stories about being in groups, how are they formed in the first place?

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1.2.4: Forming Groups

Sometimes we join a group because we want to. Other times, we might be assigned to work in groups in a class or at work. Either way, Lumsden, Lumsden and Wiethoff give three reasons why we form groups. First, we may join groups because *we share similar interests or attractions* with other group members. If you are a certain major in college, chances are you share some of the same interests as others in your class groups. Also, you might find yourself attracted to others in your group for romantic, friendship, political, religious or professional reasons. On our campus, our majors have formed the Communication Club to bring together students in the major. A second reason we join groups is called **drive reduction**. Essentially, we join groups so *our work with others reduces the drive to fulfill our needs by spreading out involvement*. As Maslow explains, we have drives for physiological needs like security, love, self-esteem, and self-actualization. Working with others helps us achieve these needs thereby reducing our obligation to meet these needs ourselves (Maslow; Paulson). If you accomplished a task successfully for a group, your group members likely complimented your work, thus fulfilling some of your self-esteem needs. If you had done the same work only for yourself, the building up of your self-esteem may not have occurred. A third reason we join groups is for **reinforcement**. We are often motivated to do things for the rewards they bring. Participating in groups provides reinforcement from others in the pursuit of our goals and rewards.

Much like interpersonal relationships, groups go through a series of stages as they come together. These stages are called *forming, storming, norming, and performing* (Tuckman; Fisher; Sherblom; Benson; Rose, Hopthrow & Crisp). Groups that form to achieve a task often go through a fifth stage called termination that occurs after a group accomplishes its goal. Let's look at each of the stages of group formation and termination.

- **Forming.** Obviously, for a group to exist and work together its members must first form the group. During the forming stage, group members begin to set the parameters of the group by establishing what characteristics identify the members of the group as a group. During this stage, the group's goals are made generally clear to members, initial questions and concerns are addressed, and initial role assignments may develop. This is the stage when group norms begin to be negotiated and established. Essentially, norms are a code of conduct which may be explicit or assumed and dictate acceptable and expected behavior of the group.
- **Storming.** The storming stage might be considered comparable to the "first fight" of a romantic couple. After the initial politeness passes in the forming stage, group members begin to feel more comfortable expressing their opinions about how the group should operate and the participation of other members in the group. Given the complexity of meeting both individual goals as well as group goals, there is constant negotiation among group members regarding participation and how a group should operate. Imagine being assigned to a group for class and you discover that all the members of the group are content with getting a C grade, but you want an A. If you confront your group members to challenge them to have higher expectations, you are in the storming stage.
- **Norming.** Back to our romantic couple example, if the couple can survive the first fight, they often emerge on the other side of the conflict feeling stronger and more cohesive. The same is true in groups. If a group is able to work through the initial conflict of the storming stage, there is the opportunity to really solidify the group's norms and get to the task at hand as a cohesive group. Norming signifies that the members of a group are willing to abide by group rules and values to achieve the group's goals.
- **Performing.** Performing is the stage we most often associate as the defining characteristic of groups. This stage is marked by a decrease in tensions, less conscious attention to norm establishment, and greater focus on the actual work at hand in order to accomplish the group's goals. While there still may be episodes of negotiating conflict and re-establishing norms, performing is about getting to the business at hand. When you are in a weekly routine of meeting at the library to work on a group project,



you are in the performing stage.

- **Terminating.** Groups that are assigned a specific goal and timeline will experience the fifth stage of group formation, termination. Think about groups you have been assigned to in college. We're willing to bet that the group did not continue once you achieved the required assignment and earned your grade. This is not to say that we do not continue relationships with other

group members. But, the defining characteristics of the group established during the forming stage have come to an end, and thus, so has the group.

Now that you understand how groups form, let's discuss the ways in which people participate in groups. Since groups are comprised of interdependent individuals, one area of research that has emerged from studying group communication is the focus on the roles that we play in groups and teams. Having an understanding of the various roles we play in groups can help us understand how to interact with various group members.

Group Communication Now

Technology is changing so many things about the ways we communicate. This is also true in group communication. One of the great frustrations for many people in groups is simply finding a time that everyone can meet together. However, computer technology has changed these dynamics as more and more groups “meet” in the virtual world, rather than face-to-face. But, what is the impact of technology on how groups function? Dr. Kiran Bala argues that “new media has brought a sea of changes in intrapersonal, interpersonal, group, and mass communication processes and content.” With group chat available on almost all social media networks and numerous technological devices, we have a lot to learn about the ways communication technologies are changing our notions of working in groups and individual communication styles.

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1.2.5: Groups Roles



Take a moment to think about the individuals in a particular group you were in and the role each of them played. You may recall that some people were extremely helpful, organized and made getting the job done easy. Others may have been more difficult to work with, or seemed to disrupt the group process. In each case, the participants were performing roles that manifest themselves in most groups. Early studies on group communication provide an overwhelming number of different types of group roles. To simplify, we provide an overview of some of the more common roles. As you study group roles, remember that we usually play more than one role at a time, and that we do not always play the same roles from group to group.

We organize group roles into four categories—task, social-emotional, procedural, and individual. **Task roles** are those that *help or hinder a group's ability to accomplish its goals*. **Social-emotional roles** are those that *focus on building and maintaining relationships among individuals in a group* (the focus is on how people feel about being in the group). **Procedural roles** are concerned with *how the group accomplishes its task*. People occupying these roles are interested in following directions, proper procedure, and going through appropriate channels when making decisions or initiating policy. The final category, **individual roles**, includes *any role "that detracts from group goals and emphasizes personal goals"* (Jensen & Chilberg 97). When people come to a group to promote their individual agenda above the group's agenda, they do not communicate in ways that are beneficial to the group. Let's take a look at each of these categories in more detail.

- **Task Roles.** While there are many task roles a person can play in a group, we want to emphasize five common ones. The **Task Leader** is the person that *keeps the group focused on the primary goal or task by setting agendas, controlling the participation and communication of the group's members, and evaluating ideas and contributions of participants*. Your associated students president probably performs the task leader role. **Information Gatherers** are those people who *seek and/or provide the factual information necessary for evaluating ideas, problem solving, and reaching conclusions*. This is the person who serves as the liaison with your professors about what they expect from a group project. **Opinion Gatherers** are those that *seek out and/or provide subjective responses about ideas and suggestions*. They most often take into account the values, beliefs, and attitudes of members. If you have a quiet member of your group, the opinion gatherer may ask, "What do you think?" in order to get that person's feedback. The **Devil's Advocate** is the person that *argues a contrary or opposing point of view*. This may be done positively in an effort to ensure that all perspectives are considered, or negatively as the unwillingness of a single person to participate in the group's ideas. The **Energizer** is the person who *functions as the group's cheer-leader, providing energy, motivation, and positive encouragement*.
- **Social-Emotional Roles.** Group members play a variety of roles in order to **build and maintain relationships in groups**. The **Social-Emotional Leader** is the person who is *concerned with maintaining and balancing the social and emotional needs of the group members and tends to play many, if not all, of the roles in this category*. The **Encourager** practices good listening skills in order to *create a safe space for others to share ideas and offer suggestions*. **Followers** are group members that do what they are told, *going along with decisions and assignments from the group*. The **Tension Releaser** is the person that *uses humor, or can skillfully change the subject in an attempt to minimize tension and avoid conflict*. The **Compromiser** is the one who *mediates disagreements or conflicts among members by encouraging others to give in on small issues for the sake of meeting the goals of the group*. What role do you find yourself most likely to enact in groups? Or, do you find you switch between these roles depending on the group?
- **Procedural Roles.** Groups cannot function properly without having a system of rules or norms in place. Members are responsible for maintaining the norms of a group and play many roles to accomplish this. The **Facilitator** acts like a traffic director by *managing the flow of information to keep the group on task*. **Gatekeepers** are those group members that attempt to *maintain proper communicative balance*. These people also serve as the points of contact between times of official group meetings. The **Recorder** is the person responsible for *tracking group ideas, decisions, and progress*. Often, a written record is necessary, thus, this person has the responsibility for keeping, maintaining, and sharing group notes. If you're the person who pulls out a pen and paper in order to track what the group talks about, you're the recorder.

- **Individual Roles.** Because groups are made of individuals, group members often play various roles in order to achieve individual goals. The **Aggressor** engages in *forceful or dominating communication to put others down or initiate conflict with other members*. This communication style can cause some members to remain silent or passive. The **Blocker** is the person that *fusses or complains about small procedural matters, often blocking the group's progress by not letting them get to the task*. They worry about small details that, overall, are not important to achieving the group's desired outcome. The **Self-Confessor** uses the group as a setting to *discuss personal or emotional matters not relevant to the group or its task*. This is the person that views the group as one that is there to perform group therapy. The **Playboy or Playgirl** shows *little interest in the group or the problem at hand* and does not contribute in a meaningful way, or at all. This is the person who does essentially no work, yet still gets credit for the group's work. The **Joker or Clown** uses *inappropriate humor or remarks that can steer the group from its mission*.

Case-in-Point

The popular sitcom *Workaholics* (2011-present) follows three college drop-outs who work in a telemarketing company and are notoriously terrible workers. Always working as a group in their shared cubicle, the three young men are all prime examples of group members who play Individual Roles: Anders as the Aggressor, Blake as the Self-Confessor, Adam as the Blocker, and all three of them as the joker or clown at one point or another. As you might guess, this group is very unproductive and ineffective.

While we certainly do not have the space to cover every role you might encounter in a group, we're sure you can point to your own examples of people who have filled the roles we've discussed. Perhaps you can point to examples of when you have filled some of these roles yourself. Important for group members to understand, are the various roles they play in groups in order to engage in positive actions that help the group along. One dynamic that these roles contribute to in the process of group communication is leadership in groups. Let's briefly examine how leadership functions in groups.

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1.2.6: Leadership In Groups

While we've examined roles we can play in groups, the role that often gets the most attention is that of the leader. Like defining communication, many people have an idea of what a leader is, but can't really come up with a good definition for the term as there are many ways to conceptualize the role of leader. One way to do this is to think of leaders in terms of their leadership styles. Let's look at three broad leadership styles to better understand the communication choices leaders can make, as well as the outcome of such choices, in a group.

First, let's visualize leadership styles by seeing them as a continuum. The position to the left (Laissez-faire) indicates a leader who exerts little to no control over a group, while the position on the right (Authoritarian) indicates a leader who seeks complete control. The position in the middle (Democratic) is one where a leader maintains a moderate level of control or influence in a group with the group's permission (Bass & Stogdill; Berkowitz).



- **Laissez-faire** is a French term that literally means "let do." This leadership style is one in which the leader *takes a laid back or hands-off approach*. For a variety of reasons, leaders may choose to keep their input at a minimum and refrain from directing a group. What do you think some reasons may be for selecting this leadership style? Perhaps a person feels uncomfortable being a leader. Perhaps a person does not feel that she/he possesses the skills required to successfully lead the group. Or, perhaps the group is highly skilled, motivated, and efficient and does not require much formal direction from a leader. If the latter is the case, then a laissez-faire approach may work well. However, if a group is in need of direction then a laissez-faire style may result in frustration and inefficiency.
- An **authoritarian leadership style** is one in which a leader attempts to *exert maximum control over a group*. This may be done by making unilateral decisions rather than consulting all members, assigning members to specific tasks or duties, and generally controlling group processes. This leadership style may be beneficial when a group is in need of direction or there are significant time pressures. Authoritarian leaders may help a group stay efficient and organized in order to accomplish its goals. However, group members may be less committed to the outcomes of the group process than if they had been a part of the decision making process. One term that you may have heard on your campus is "shared-governance." In general, faculty do not like working in groups where one person is making the decisions. Instead, most faculty prefer a system where all members of a group share in the leadership process. This can also be called the democratic style of leadership.
- The **democratic style of leadership** falls somewhere in the middle of laissez-faire and authoritarian styles. In these situations, *the decision-making power is shared among group members, not exercised by one individual*. In order for this to be effective, group members must spend considerable time sharing and listening to various positions and weighing the effects of each. Groups organized in this fashion may be more committed to the outcomes of the group, creative, and participatory. However, as each person's ideas are taken into account, this can extend the amount of time it takes for a group to accomplish its goals.

While we've certainly oversimplified our coverage of the complex nature of group leaders, you should be able to recognize that there are pros and cons to each leadership style depending on context. There is not one right way to be a leader for every group. Effective leaders are able to adapt their leadership style to fit the needs of the group. Furthermore, as a group's needs and members change over time, leadership styles can accommodate natural changes in the group's life cycle. Take a moment to think of various group situations in which each leadership style may be the most and the least desirable. What are examples of groups where each style of leadership could be practiced effectively?

Group Norms

Every group in which we participate has a set of norms like we discussed in the "norming" stage. Each group's rules and norms are different, and we must learn them to be effective participants. Some groups formalize their norms and rules, while others are less formal and more fluid. Norms are the recognized rules of behavior for group members. **Norms influence the ways we communicate with other members, and ultimately, the outcome of group participation.** Norms are important because, as we highlighted in the "norming" stage of group formation, they are the defining characteristics of groups.



Brilhart and Galanes divide norms into two categories. **General norms** "direct the behavior of the group as a whole" (130). Meeting times, how meetings run, and the division of tasks are all examples of general norms that groups form and maintain. These norms establish the generally accepted rules of behavior for all group members. The second category of norms is role-specific norms. **Role-specific norms** "concern individual members with particular roles, such as the designated leader" (130).

Not only are there norms that apply to all members of a group, there are norms that influence the behaviors of each role. Consider our brief discussion on leadership. If a group's members are self-motivated, and do not need someone imposing structure, they will set a norm that the group leader should act as a laissez-faire or democratic leader rather than an authoritarian leader. Violation of this norm would most likely result in conflict if leaders try to impose their will. A violation like this will send a group back to the "storming" stage to renegotiate the acceptable norms of the group. When norms are violated, group members most often will work to correct the violation to get the group back on task and functioning properly. Have you ever been in a group in which a particular group member did not do the task that was assigned to them? What happened? How did the group handle this situation as a whole? What was the response of the person who did not complete the task? In hindsight, would you have handled it differently? If so, how?

As groups progress through the various stages, and as members engage in the various roles, the group is in a continual process of decision making. Since this is true, it makes sense to ask the question, "How is it that groups make decisions?"

Contributions and Affiliations

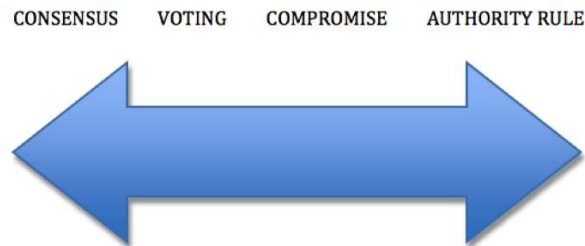
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1.2.7: Decision Making In Groups

When groups need to get a job done they should have a method in place for making decisions. The decision making process is a norm that may be decided by a group leader or by the group members as a whole. Let's look at four common ways of making decisions in groups. To make it simple we will again use a continuum as a way to visualize the various options groups have for making decisions. On the left side are those methods that require maximum group involvement (consensus and voting). On the right are those methods that use the least amount of input from all members (compromise and authority rule).



The decision-making process that requires the most group input is called **consensus**. To reach consensus *group members must participate in the crafting of a decision and agree to adopt it*. While not all members may support the decision equally, all will agree to carry it out. In individualistic cultures like the U.S., where a great deal of value is placed on independence and freedom of choice, this option can be seen by group members as desirable, since no one is forced to go along with a policy or plan of action to which they are opposed. Even though this style of decision making has many advantages, it has its limitations as well—it requires a great deal of creativity, trust, communication, and time on the part of all group members. When groups have a hard time reaching consensus, they may opt for the next strategy which does not require buy-in from all or most of the group.

Group Communication and You

Okay, you're a Communication major and this whole idea of working in groups really appeals to you and seems to come naturally. But perhaps you're not a Communication major and you're thinking to yourself that your future career isn't really going to require group or team work. Well, you might want to think again. Forbes magazine released an article titled *The Ten Skills Employers Most Want in 2015 Graduates* which stated that technical knowledge related to the job is not nearly as important as effective teamwork and communications skills. In fact, the top three skills listed include, 1) ability to work in a team structure, 2) ability to make decisions and solve problems, and 3) ability to communicate verbally with people inside and outside an organization. Even non-Communication majors need to develop effective group communication skills to succeed at work. Even non-Communication majors need to develop effective group communication skills to succeed at work.

Voting by majority may be as simple as having 51% of the vote for a particular decision, or may require a larger percentage, such as two-thirds or three-fourths, before reaching a decision. Like consensus, voting is advantageous because everyone is able to have an equal say in the decision process (as long as they vote). Unlike consensus, everyone may not be satisfied with the outcome. In a simple majority, 49% of voters may be displeased and may be resistant to abide by the majority vote. In this case the decision or policy may be difficult to carry out and implement. For example, our campus recently had a department vote on whether or not they wanted to hire a particular person to be a professor. Three faculty voted yes for the person, while two faculty voted no. Needless to say, there was a fair amount of contention among the professors who voted. Ultimately, the person being considered for the job learned about the split vote and decided that he did not want to take the job because he felt that the two people that voted no would not treat him well.

Toward the right of our continuum is **compromise**. This method often carries a positive connotation in the U.S. because it is perceived as fair since *each member gives up something, as well as gains something*. Nevertheless, this decision making process may not be as fair as it seems on the surface. The main reason for this has to do with what is being given up and obtained. There is nothing in a compromise that says these two factors must be equal (that may be the ideal, but it is often not the reality). For individuals or groups that feel they have gotten the unfair end of the bargain, they may be resentful and refuse to carry out the compromise. They may also foster ill will toward others in the group, or engage in self-doubt for going along with the compromise in the first place. However, if groups cannot make decisions through consensus or voting, compromise may be the next best alternative.

At the far right of our continuum is decision by **authority rule**. This decision-making process *requires essentially no input from the group, although the group's participation may be necessary for implementing the decision*. The authority in question may be a member of the group who has more power than other members, such as the leader, or a person of power outside the group. While this method is obviously efficient, members are often resentful when they feel they have to follow another's orders and feel the group process was a façade and waste of valuable time.

During the decision making process, groups must be careful not to fall victim to groupthink. **Groupthink** happens when *a group is so focused on agreement and consensus that they do not examine all of the potential solutions available to them*. Obviously, this can lead to incredibly flawed decision making and outcomes. Groupthink occurs when members strive for unanimity, resulting in self-deception, forced consent, and conformity to group values and ethics (Rose, Hopthrow & Crisp). Many people argue that groupthink is the reason behind some of history's worst decisions, such as the Bay of Pigs Invasion, The Pearl Harbor attack, The North Korea escalation, the Vietnam escalation, and the Bush administration's decision to go to war with Iraq (Rose, Hopthrow & Crisp). Let's think about groupthink on a smaller, less detrimental level. Imagine you are participating in a voting process during a group meeting where everyone votes yes on a particular subject, but you want to vote no. You might feel pressured to conform to the group and vote yes for the sole purpose of unanimity, even though it goes against your individual desires.

As with leadership styles, appropriate decision making processes vary from group to group depending on context, culture, and group members. There is not a "one way fits all" approach to making group decisions. When you find yourself in a task or decision-making group you should consider taking stock of the task at hand before deciding as a group the best ways to proceed.

Group Work and Time

By now you should recognize that working in groups and teams has many advantages. However, one issue that is of central importance to group work is time. When working in groups time can be both a source of frustration, as well as a reason to work together. One obvious problem is that it takes much longer to make decisions with two or more people as opposed to just one person. Another problem is that it can be difficult to coordinate meeting times when taking into account people's busy lives of work, school, family, and other personal commitments. On the flip side, when time is limited and there are multiple tasks to accomplish, it is often more efficient to work in a group where tasks can be delegated according to resources and skills. When each member can take on certain aspects of a project, this limits the amount of work an individual would have to do if he/she were solely responsible for the project.

For example, Alex, Kellsie and Teresa all had a project to work on. The project was large and would take a full semester to complete. They had to split up the amount of work equally to each person as well as based on skills. The thought of doing all the work alone was daunting in terms of the required time and labor. Being able to delegate assignments and work together to achieve a professional result for their project indicated that the best option for them was to work together. In the end, the group's work produced different results and views

that you wouldn't have necessarily come to working alone. On the flip side, imagine having to work in a group where you believe you could do just the same on your own. When deciding whether or not to work in groups, it is important to consider time. Is the time and effort of working in a group worth the outcome? Or, is it better to accomplish the task as an individual?

Groups and Technology and Social Media

Social media and technology are changing the ways we communicate in groups. There is no doubt that technology is rapidly changing the ways we communicate in a variety of contexts, and group communication is no exception. Many organizations use computers and cell phones as a primary way to keep groups connected given their ease of use, low cost, and asynchronous nature. In fact, it's likely that your course web pages also have "group forums" for class groups to deal with the complexities of finding times to meet. In fact, the group that worked on this chapter used Google Docs to have live chats online, transfer documents back and forth, and form messages to achieve the group's goals—all without ever having to meet in person. As you enter the workforce, you'll likely find yourself participating in virtual groups with people who have been brought together from a variety of geographical locations.

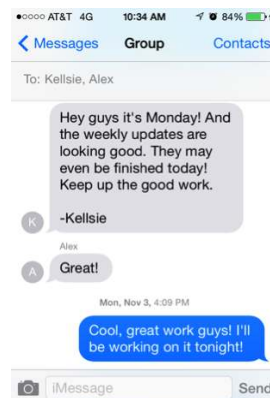
Group Communication and You

Today, we know that social media has the power to bring people together and drive change. Sports fans flock to Twitter and other social media outlets to follow their favorite teams and athletes. Less than 5 percent of TV is sport, but 50 percent of what is tweeted is about sports. And if there was any thought that this phenomenon was only for Americans that is quickly debunked by the worldwide usage statistics.

"The power to create the global sports village and encourage the next generation of pro-social media sports fans is at our fingertips." -Adam C. Earnhardt

While communication technologies can be beneficial for bringing people together and facilitating groups, they also have drawbacks. When we lack face-to-face encounters, and rely on asynchronous forms of communication, there is greater potential for information to be lost and messages to be ambiguous. The face-to-face nature of traditional group meetings provides immediate processing and feedback through the interaction of group members. When groups communicate through email, threads, discussion forums, text messaging, etc., they lose the ability to provide immediate feedback to other members. Also, using communication technologies takes a great deal more time for a group to achieve its goals due to the asynchronous nature of these channels.

Case in Point



This screen shot is from an iMessage group chat between the group that is responsible for this chapter's content. Texting helped our group stay connected, updated, and motivated without having to meet face-to-face.

Nevertheless, technology is changing the ways we understand groups and participate in them. We have yet to work out all of the new standards for group participation introduced by technology. Used well, technology opens the door for new avenues of working in groups to achieve goals. Used poorly, technology can add to the many frustrations people often experience working in groups and teams.

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1.2.8: Group Communication Summary

Summary

We participate in groups and teams at all stages and phases of our lives, from play groups, to members of an athletic team, to performing in a band, or performing in a play. We form groups based on personal and professional interests, drive reduction, and for reinforcement. Through group and team work we can save time and resources, enhance the quality of our work, succeed professionally, or accomplish socio-political change.

As you recall, a group is composed of three or more people who interact over time, depend on each other, and follow shared rules and norms. A team is a specialized group which possesses a strong sense of collective identity and compatible and complimentary resources. There are five general types of groups depending on the intended outcome. Primary groups are formed to satisfy our long-term emotive needs. Secondary groups are more performance based and concern themselves with accomplishing tasks or decision making. Personal growth groups focus on specific areas of personal problem solving while providing a supportive and emotionally positive context. Learning groups are charged with the discovery and dissemination of new ideas while problem solving groups find solutions.

Once a group comes together they go through typical stages (forming, storming, norming, performing, and terminating) to develop roles, create a leadership strategy, and determine the process for decision making. While numerous specific group roles exist, the four categories of roles include: task, social-emotional, procedural, and individual roles. It is likely that members will occupy multiple roles simultaneously as they participate in groups.

There are three broad leadership styles ranging from least to most control—laissez faire, democratic, and authoritarian. Also related to power and control are options for decision making. Consensus gives members the most say, voting and compromise may please some but not others, and authority rule gives all control to the leader. None of the options for leadership styles and decision making are inherently good or bad—the appropriate choice depends on the individual situation and context. It is important for groups not to become victims of groupthink as they make decisions.

New technologies are continually changing how we engage in group communication. The asynchronous nature of communication technologies can facilitate group processes. However, they also have the potential to slow groups down and make it more difficult to accomplish group goals.

DISCUSSION QUESTIONS

1. What are the differences between the terms “team” and “group?” Write down a team you have been a part of and a group you have been a part of. In what ways were they effective or not effective?
2. Review all the different types of group roles. Reflect back on a time when you worked in a group and discuss the role(s) you played. If there were any individuals in this group that prevented the group’s progress, identify their role and explain why it was problematic.
3. Thinking back to groups that you have been involved with in the past, which types of groups had the most effective leader(s) and what were the qualities of those leaders that made them so strong?
4. What are the potential strengths of group discussions? What are the potential limitations of group discussions? What are some strategies to enhance a group’s cohesion?
5. Reflect back on a time when you were working on a group project in class. Discuss each stage of development (forming, storming, norming, performing, and terminating) as it applied to this group.
6. How were/are decisions made in your family? Has the process changed over time? What kinds of communication surround the decision making?

KEY TERMS

- activity groups
- aggressor
- authoritarian
- authority rule
- blocker
- brainstorming
- climate

- cohesiveness
- collectivist
- common goals
- compromise
- consensus
- democratic
- devil's advocate
- drive reduction
- encourager
- energizer
- facilitator
- followers
- forming
- gatekeepers
- general norms
- group
- groupthink
- individualistic
- individual roles
- information gatherers
- interaction
- interdependence
- interests/attraction
- joker/clown
- laissez-faire
- leadership
- learning groups
- norming
- norms
- opinion gatherers
- performing
- personal growth groups
- playboy/playgirl
- polarization
- power
- power-from-within
- power-over
- power-with
- primary groups
- problem solving groups
- procedural roles
- promulgation
- recorder
- reinforcement
- role-specific norms
- secondary groups
- self-confessor
- shared norms
- social-emotional roles
- social-emotional leader
- solidification
- storming

- synergy
- task leader
- task roles
- teams
- tension releasers
- terminating
- voting

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CHAPTER OVERVIEW

2: Untitled Chapter 2

[2.1: Counselor's Manual for Relapse Prevention with Chemically Dependent Criminal Offenders](#)

[2.2: Your Ultimate Group Therapy Guide](#)

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2.1: Counselor's Manual for Relapse Prevention with Chemically Dependent Criminal Offenders

Group counseling has proved to be the most effective way of treating chemical dependency. This chapter explains how to do group counseling. Patients in chemical dependency treatment programs learn best in group counseling, where patients learn about themselves by interacting with others. They also come to understand that they are not alone in their problems. In addition, they learn social and communication skills that allow them to make better use of self-help programs such as Alcoholics Anonymous and Narcotics Anonymous.

How Is Group Work Different From Individual Counseling?

Group counseling and individual counseling are both important tools for treating chemical dependency. Group counseling uses many of the same intervention strategies as individual counseling. There are, however, some important distinctions between the two modalities. A common mistake for beginning group counselors is to focus an entire group meeting on one patient, while the others in the group simply look on.

Group counseling is different from individual counseling in the following ways:

- Group counseling focuses on the present; the here and now. In group counseling, patients do not delve into long accounts of personal history that preceded the problems of chemical dependency. Group counseling provides a forum to understand current behavior, to learn about chemical dependency, to discuss new ways of behaving, to learn new ways to solve problems, and to develop relapse prevention skills.
- Group counseling makes use of the interactive process within the group. That is, the counselor focuses on how the group members act toward one another, communicate with one another, and how they behave in the group.
- The counselor and group members offer individuals feedback about their behavior. In individual counseling patients simply disagree with their counselor. In group counseling the counselor's feedback is combined with positive peer feedback from the group members. This makes messages more powerful.
- The group provides a place for the counselor to help individuals practice new skills such as problem solving, communication, and managing stress.

In group counseling, the counselor uses a peer group to influence individual patients and change behavior in a positive way.

Group Counseling Theory

Stages of Group Development

When a group first begins, counselors and group members alike will feel very uncomfortable. The members may not know the counselor or one another. As people become familiar with the group, feelings and behavior begin to change. These changes follow predictable patterns. In fact, groups have a clear developmental life cycle, that is, a group goes through different stages. As the group leader gains experience, he or she learns to anticipate these changes and work with them.

There are many models for the stages of group development. The following is a composite of several models:

- Stage 1—Preaffiliation
- Stage 2—Power and control
- Stage 3—Intimacy
- Stage 4—Differentiation
- Stage 5—Separation.

In the preaffiliation stage, members feel uncomfortable, anxious, or fearful with the newness of the experience. In this stage, members look to the leader for direction. Initially, the group should be leader-focused, with the leader helping members adjust to the new experience.

Once group members are more comfortable, it is predictable that they will challenge the authority of the leader and will pursue power and control. It is important for the leader to remember that this is a normal style in the group's development, not unlike the challenges that face the parents of an adolescent. This phase may be uncomfortable, with group members expressing anger and frustration. The leader should be careful not to personalize these challenges to authority. The leader should be consistent, avoid fighting with the group, and allow the group to become more autonomous without sacrificing his or her position of authority.

In the next stage, some degree of intimacy is established. It is very important for the leader to move members to a common level of intimacy before allowing too much self-disclosure by the group members. The setting and type of the group will determine the overall level of intimacy. As members feel safer in the group, they can better engage in activities and take risks necessary for change. At this stage, the leader can give less direction, allowing the members to work together more spontaneously and more independently.

Differentiation is the stage at which members have a strong sense of identification with the group and feel trusting. This is the most productive stage of group development.

Finally, at the point of termination or separation, members experience a range of feelings and display a range of behaviors in anticipation of leaving the group. It is important to remember that chemically dependent people typically have experienced a lot of loss over their lifetimes. Many have lost family members and friends to violence and illness. They do not handle the ending of relationships well. Termination of the group or loss of a group member presents an important opportunity to deal with this problem. The leader should begin to prepare the group for ending well in advance and do so gradually. The leader can expect members to use denial or to regress. It is important to predict these behaviors and to identify them as they occur.

These stages of group development are very predictable. Virtually all groups go through them. However, depending upon the circumstances a group may regress to an earlier stage at any time. For example, if a group adds new members, the level of intimacy will decrease. The group may return to a stage of preaffiliation. It is hard to predict how long a group will stay in a particular stage of development. The type of group (i.e., mandatory or voluntary), the setting (i.e. institution or community), and other factors can all influence the process. With experience, the group leader develops the skills to promote the group developmental process or alter.

Communication in Groups: Content and Interactive Process

The terms "content" and "interactive process" refer to the patterns of communication among group members. "Content" refers to the *substance* of a communication. The content is the subject matter, including issues, questions, or problems on which the group is focused. "Interactive process" refers to *how* members communicate and act with one another. The process includes not only the spoken words, but also the nonverbal messages expressed by tone of voice, posture, and facial expression. Process provides the "present focus" or "here and now" raw material for group treatment.

The content of a group meeting sometimes symbolizes the group process. In the same way a client might talk about "a friend who has a problem," group members may talk about prior events and issues that reflect current experiences. Often as group leaders, we get caught up in the content. We are very interested in the what, when, where, who, how, and why. In group counseling, this content has relevance in a way that can be different from its relevance in individual counseling.

The Counselor as Group Leader

Many techniques used in group counseling are similar to those used in individual counseling. The general approach of the group leader, however, must work to create a group culture that focuses on the "here and now" behavior. An active and dynamic approach along with an empathic style are needed to do this.

The group leader's focus should:

- encourage group and individual recovery
- teach members about chemical dependency, recovery, and relapse prevention
- build members' self-esteem.

The group leader's approach should:

- be empathic
- instill hope
- model desired behaviors
- treat all members consistently, equally, and fairly
- be active and directive
- use appropriate interventions to keep the group moving.

The group leader should:

- maintain control in a nonauthoritative way
- be firm but not punitive
- be assertive in setting limits

- provide appropriate rewards (activities, trips, etc.) to the group.

Planning for Group Work

Logistics

All logistical arrangements should be planned well in advance of beginning the group. In order for the preplanning to go smoothly, group counselors should seek the support of appropriate administrative and support staff. Establish the following before getting started.

Group Size

Groups typically range in size from 6 to 12 people. The size should be determined by such factors as the type of group and the capacity of the patients. "Capacity" refers to the level of individual functioning. Can the patient concentrate, focus, and pay attention? Some substance abusers, particularly those in the early stages of recovery, cannot make use of all their mental functions. Others may have mental/emotional problems that interfere with these abilities. Low-functioning individuals will need a smaller group. Educational groups can handle more members, whereas process oriented-groups should be smaller.

Time

Time is an important boundary. The length of group sessions should be preplanned if the group is to be time limited. A schedule of sessions should be established that considers holidays and other commitments.

Sessions should be of equal length. The ideal length depends on the capacity of the patients, the setting, and the type of group. More functional patients can handle longer sessions than less functional or younger patients. The materials presented in this manual are intended for two-hour group sessions.

Once the time boundaries have been established, it is very important to begin and end group sessions on time.

Space

The space chosen for group meetings will make a statement about the importance given to this activity. The space should be psychologically positive and provide a safe environment for the emotional risks that go with treatment. The space should be well lighted, well ventilated, and an appropriate size for the size of the group. A private location that is accessible, free from interruptions, and physically safe should be chosen.

Types of Groups

Different types of groups serve different purposes. The following is a review of some options to help you decide what type of group is most practical and useful for the setting.

Mandatory or Voluntary Group?

You might assume that voluntary groups are best, but research and practice indicate that both voluntary and mandatory groups have their advantages and disadvantages.

Mandatory groups ensure that members will attend. With regular attendance the group process can develop with little disruption. Unfortunately, mandatory requirements often increase hostility and resistance and intensify denial. No one likes to be told they must go to a counseling group, and few counselors like being confronted with such hostility, particularly by a group of eight or more people.

When the counselor is well prepared, the situation can be managed. Patients will attempt to engage you in battle. The best tactic is to avoid these battles. One way to do so is to join with the group by saying something like, "You have to be here and I have to be here. I understand and appreciate your anger but it is not my fault. How can we both make the best of things?" Offering concrete rewards for cooperation may also help. Setting rules for attendance can eliminate overt resistance but seldom reduces passive resistance.

The disadvantages of the mandatory group become the advantages of the voluntary group. Members of voluntary groups identify with one another, denial is less potent, there is less hostility, and one can move on more quickly to group goals. However, the voluntary group does not have some major disadvantages. When participation is voluntary, members often find excuses to be absent when there is pressure on them to face problems. Without a "captive" audience, leaders find that it is hard to ensure member attendance and that it is difficult for the group process to evolve with absent members.

Which type of group is best? Research indicates that mandatory treatment works as well as voluntary treatment with substance abusers. In a criminal justice setting, required attendance can be useful for all.

Open or Closed Group Membership?

One issue to be decided in advance is whether or not to add members after the group has started. The terms "open membership" and "closed membership" are used to describe the two options.

Open membership can reach more clients and is easier to keep going over time because lost members can be replaced. However, adding new members can cause a loss of group intimacy and cohesiveness. Development may regress. Although this may not be ideal, depending on the goals of group, adaptations can be made. Closed membership allows for greater individual progress but is impractical in some settings.

Time-Limited or Open-Ended Group?

It may be practical to place a time limit on a group depending on the patients' stage of recovery. This way patients graduate together to another group with another specified goal. If the setting allows, an open-ended, closed membership group can be ideal. Such a group can achieve high levels of intimacy and differentiation that allow for greater risk taking. The goals can advance while the membership remains the same.

Educational Groups?

For patients to succeed in recovery, they must learn certain things about chemical dependency. This information helps them to cope with the challenges of recovery and avoid relapse. Educational groups also help *engage* the client in treatment and recovery. The overt or covert expectation of individual or group therapy is "change." People find this threatening. An educational group is much less threatening because it is easier to "learn" than to "change."

Educational sessions can be offered in 60- to 90-minute blocks. The educational sessions should offer basic information on

- Chemical dependency as a bio/psycho/social disease
- The recovery process
- Symptoms that appear after beginning abstinence
- Relapse warning signs
- Recovery planning.

These sessions should consist of a lecture and an exercise, presented with media supplements. Some programs have an educational curriculum available, and such programs may be available through your State Alcohol and Drug Abuse Agency. One prepackaged educational program that can be ordered is the *Staying Sober Educational Modules* (see the bibliography).

Group Goals and Principles

It is important for the group leader to be clear about what is to be accomplished in the group. It is best to have a written goal with a step-by-step plan for reaching the goal. Having both a written goal and a plan will help to keep the group on track.

The goals of group treatment with addicted patients should be:

Self-assessment. The patients should be able to talk about and understand the meaning of different exercises to their recovery.

Communication training. The group leader should teach patients basic techniques for talking about their thoughts, feelings, and reports of life events. This training should focus on teaching patients how to reveal things about themselves and how to give and receive feedback.

Cognitive restructuring. Patients should relearn how to think so that they can accurately examine and report information and understand how it pertains to their recovery.

Effective counseling. Group treatment should teach patients how to identify, express and self-regulate their emotions and moods.

Memory retraining. Treatment should help patients restore short-term and long-term memory.

Treatment monitoring. Group sessions should provide a vehicle for monitoring and holding the patient accountable for progress and problems encountered when pursuing treatment goals.

Support. The group leader should provide peer and professional support throughout the recovery process.

Opportunity for dialogue. Group sessions should give the patient a chance to talk about recovery issues in a supportive environment where feedback from and discussion with people both more and less advanced in the recovery are available.

Involvement of others in problem solving. The group process should involve the patient in problem solving with other recovering people. The group leader can tell patients that others can and will help in problem solving if they are allowed to, that they too are capable of helping others, and that they can help themselves by helping others.

The principles of group counseling for patients recovering from addictions should be:

Addiction groups. In order to be successful, groups must consist of only recovering alcoholics and drug addicts.

Group treatment goals with addicted patients. The addicted patient is suffering from chemical dependency. This is an illness causing specific physical, mental and social impairments. Group treatment must be directed at helping the patient with these impairments.

Structured and directive group process. The group process should be structured rather than free-floating. Patients must focus on concrete, specific problem solving relating to accepting their addiction and achieving a comfortable recovery. Feelings should be dealt with in the context of these concrete problems. All problems dealt with in the group must be related to recovery from chemical dependency.

The role of the group counselor. The group counselor should be directive, yet permissive and supportive. The counselor is responsible for establishing and maintaining direction for each patient and for the group as a whole. The group counselor gives direction and supervision to the group. He or she must provide a consistent group format; set the pace of the group and see that it is maintained; assign, follow, and review assignments; and manage group problems as they develop.

The abstinence goal. The first focus of group treatment should be for each patient to establish and maintain abstinence from alcohol and mood-altering drugs. This goal of recovery involves the identification of concrete problems and situations that could jeopardize abstinence, the development of specific plans for managing these problems, and the completion of skills training and assignments designed to develop skills in coping with these problems and situations.

Reliance on group support. Patients need to develop a strong substitute dependency to replace their old dependency on alcohol and drugs. Patients will tend to develop a strong dependency on the counselor as this substitute dependency. Group treatment should be used to transfer this dependency from the counselor to the group. Group counseling for addicted patients should be designed to support the patient's ongoing involvement in AA, NA, and other support groups. It should also focus on building strong, positive, supportive relationships among the group members.

Admission and discharge criteria. There should be specific admission criteria that describe the type of patient that is appropriate for treatment in group counseling. There should also be specific discharge criteria that describe when a patient is ready to responsibly "graduate" from the group.

Issues that are inappropriate for group treatment. There are certain issues that are best dealt with individually. This is due to the need for extreme confidentiality or a patient's inability to deal with the issues in a group setting. Care needs to be taken, however, not to support a patient's continuing denial by allowing him or her to avoid talking about routine recovery issues in group sessions.

Role modeling by the counselor. The counselor should model the behaviors that he or she expects from the patients.

Supportive counseling. The early efforts of the group counselor should be directed toward allying himself or herself with the addicted patient's needs rather than with attacking defenses. Addicted patients need basic support, education, communication training, and direction in recovery. These should be provided with support rather than harsh confrontation.

Group involvement. Eighty percent of the benefits of group treatment comes from becoming actively involved in utilizing the group process to help other group members to recover. This involvement interrupts chemically dependent self-centered behavior and provides training in the processes of problem solving and recovery. Many patients will automatically identify and discover solutions to problems in their lives by helping other patients cope with similar problems. Only 20 percent of the benefits of group counseling comes from working on personal problems.

Note taking and tape recording in group. Addicted patients suffer from severe memory impairments. It is recommended that all patients take notes on important issues. Patients can also tape record portions of the group sessions where they work on an issue and receive feedback. Listening to these tapes later often speeds up the counseling process.

The intoxicated patient in group treatment. It is unproductive to allow a patient to attend group sessions while actively intoxicated with alcohol or drugs. The patient should be asked to leave the group and an individual appointment should be made to motivate

the patient to enter appropriate detoxification treatment.

Rules and Contracting

Contracting is a tool that many groups use to help get members to attend meetings and follow rules. Because it is very important that all members agree to the requirements and rules of the group, a document can be written up and copied for each member and the group leader. Each member and the leader will sign this contract. The contract sets forth the day or dates of meetings, time, location and group rules.

Clearly stated and enforced rules are critical for a successful group. They can free members to deal with recovery issues. For example, when a rule of no violence is clearly stated and enforced, it allows the patient to feel and express anger, knowing that the group will not allow any one person to get out of control. Rules also offer limits to patients who have very few internal controls and who cannot set their own limits. Many substance abusers grew up in situations that did not teach them controls and limits. Establishing and enforcing group rules can help correct this.

Use the following guidelines in setting group rules.

- Do not make a rule that you or the agency cannot enforce.
- All rules must be enforced fairly and anytime they are violated.
- Rules should be clear and understood by all.
- Many substance abusers have memory problems; therefore, rules should be restated periodically and whenever a new members joins the group.

Group Rules

The following rules are designed to be used as part of the problem-solving group process.

- You can say anything you want, any time you want to say it. Silence is not a virtue in this group and can be harmful to your recovery.
- You can refuse to answer any question or participate in any activity except the basic group responsibilities. The group cannot force you to participate, but group members do have the right to express how they feel about your silence or your choice not to get involved.
- What happens in the group stays among the members with one exception: Counselors may consult with other counselors in order to provide more effective treatment.
- No swearing, putting down, physical violence, or threat of violence.
- No dating, romantic involvement, or sexual involvement among the members of the group. Such activities can sabotage the treatment of those involved and others. If such involvements develop, members should bring it to the attention of the group or individual counselor at once.
- Anyone who decides to leave group treatment must tell the group in person prior to termination.
- Group sessions are 2 hours in duration. Patients should be on time and plan on not leaving the session before it is over. Smoking, eating, and drinking are not allowed in group sessions.

Responsibilities of patients in the group include the following:

- Listen to other group members' problems.
- Ask questions to help clarify problems or proposed solutions.
- Give feedback about what you think and feel about a problem and the personal strengths you see in the person that will help him or her solve the problem. Also give feedback about the weaknesses you see that may set the person up to fail to solve the problem.
- Share personal experiences with similar problems when appropriate. Self-disclosure must be carefully managed to keep the primary focus on the patient who is working on the issue.

Problem-Solving Group Counseling Format

The Preparation Session

Before the group session begins, the counselors must prepare. Counselors meet as a group. A brief written description of each patient (a "thumbnail" sketch) is presented, and the patient's progress is reviewed. An attempt is made to predict the assignments and problems that patients will present.

The Opening Procedure (5 Minutes)

During the opening procedure, the counselor sets the climate for the group, establishes leadership, and helps patients warm up to the group process.

- The counselor enters the group room. He or she makes sure that the room is set up with a circle of upright chairs arranged close enough for the members to touch each other. The counselor greets each member informally by talking to them before the group starts.
- The counselor asks the members to touch the person on either side and while doing this make eye contact with each person in the group. He asks them to make sure the other person sees them by nodding or giving another response.
- The counselor completes a centering technique (see Counseling Techniques). This is designed to get the patient in touch with himself or herself and leave nonrelated problems outside the group room.
- The counselor then takes attendance. During the attendance procedure, the counselor makes eye contact with each patient, engages in a brief social greeting, and tries to get an idea of each patient's attitude and mood before going on.

Reactions to Last Session (15 Minutes)

A reaction is a brief description of (1) what each group member thought during the last group session, (2) how the group member felt during the last group session, and (3) identification of the three persons who stood out from the last session and why they were remembered.

All group members are required to give a reaction to the last session. This accomplishes a variety of goals:

- It forces each patient to talk in the first phase of the group session.
- It breaks the tendency toward isolation and self-centeredness by forcing the patient to notice and comment on at least three other group members. This reaction forces group involvement.
- It provides training in basic communication and on how to give feedback.
- It provides feedback to other group members about who stood out from the last session and why.
- It puts pressure on group members to recall important events from previous group sessions. As a result, it serves as a memory training device.
- It tests a group member's motivation. Members who refuse to give reactions or repeat what others say generally have problems cooperating with other aspects of treatment.
- It provides an opportunity for the counselor to reflect on the last group and compare his or her personal memory with the group members' memories.

It is important to remember that a reaction is a one-way communication. Other group members are not permitted to comment on the reactions. If someone is upset by what another group member says, it is that person's responsibility to volunteer to work on the issue when the agenda is set.

A reaction is also a no-fault communication. There are no right or wrong reactions. The only feedback the counselor and other group members generally give is on the format and completeness of the reaction. In other words, the group member is reporting on his or her thoughts, feelings, and at least three persons who stood out to them in the last session.

A typical reaction should have three parts:

- What I thought about during the last group session.
- How I felt during the last group session.
- Three people that stood out to me in the last group session.

A counselor must help patients by coaching their responses.

Typical problems that patients will have are

- They will say what they thought about or felt about the last session, instead of what they thought or felt during the last session.

Example:

Patient—I thought last week's session was good. (Incorrect)

Counselor—You misunderstand. What I would like you to do is tell what conversations or pictures went on in your head.

Patient—What I thought about during last week's session was how my drinking and drug use has affected my life. (Correct)

- They will confuse thoughts and feelings.

Example:

Patient—I think I was angry. (Incorrect)

Counselor—You felt angry? (Explain the difference between a thought and a feeling.)

Patient—I felt angry. I was thinking about going to jail. (Correct)

- They will talk about a group member instead of to them.

Example:

Patient—Joe stood out because his life history was a lot like mine. (Incorrect)

Counselor—Please say this again and this time speak directly to Joe. Look at him and say, "Joe, you stood out because . . ."

Patient—Joe, you stood out because your life history was a lot like mine. (Correct)

Examples of Good Responses. A typical reaction made by a group member to the last group might be as follows:

- I thought a lot about how I deal with anger and frustration. There was a lot of good feedback when I talked about my problem.
- I had a feeling of accomplishment as I worked on my problems. I was surprised. I got excited instead of depressed for the first time in a long time.
- Joe, you stood out to me because you understood what I was talking about.
- Mary, you stood out to me because you told me you cared. I'm not sure if I believe you. A part of me thought you were telling the truth and I felt good. Another part of me said, "Why should she care—no one else does."
- Pete, you stood out because you did not seem to pay attention to me when I was talking.

Learning To Give Good Reactions

It takes time for the average person to learn how to give good reactions in group session. This learning takes place as a result of instruction and imitation. The counselor and other group members should explain the components of a good reaction to each new group member. A written handout should be provided that describes the components of a reaction and gives examples.

The group member will also learn by observing and imitating the reactions of other group members. Counselors can speed up this progress by acknowledging good reactions. This is done by saying, "Good" or another positive response to encourage and reward the person. The counselor gives positive feedback for doing the reaction correctly, not based on agreement with the content of what the person says.

Report on Assignments (10 minutes)

Assignments are exercises that patients are working on in their workbook or in addition to their workbook. Additional assignments are often given to help a group member solve a problem that is being worked on in the group. Some of these assignments will be completed in group, and others will need to be completed in between group sessions. Immediately following reactions, the counselor will ask all group members who have received assignments to briefly answer six questions.

- What was the assignment and why was it assigned?
- Was the assignment completed? If not, what happened when you tried to do it?
- What was learned from the completion of the assignment?
- What feelings and emotions did you experience while completing the assignment?
- Did any issues surface that require additional work in group?
- Is there anything else that you want to work on in group today?

Patients should be asked to rate how important their assignment or problems are in the group session by labeling them with a number from 1 to 10, with 1 being not very important and 10 being extremely important.

Setting the Agenda (3 Minutes)

After all assignments have been reported on, the group counselor will identify all persons who want to work, and announce who will work and in what order. Group members who do not have time to present their work in this group session will be first on the agenda in the following group session. It is best to not plan on over three patients working in any group session.

The Problem-Solving Group Process (70 Minutes)

The problem solving group process is designed to allow patients to present issues to the group, clarify these issues through group questioning, receive feedback from the group, receive feedback from the counselor (if appropriate), and develop assignments for continued progress.

The problem solving process is guided in two ways. A series of exercises are assigned and then processed in group. Special problems that come up are discussed by the group. One goal of group counseling is to teach problem solving skills that will enable the recovering patient to handle difficult situations when they arise.

When dealing with problems that are not assignments, a standard problem solving process is recommended. This process consists of the following steps:

Step 1: Problem Identification.

First, have the members ask questions to identify what is causing difficulty. What is the problem?

Step 2: Problem Clarification.

Encourage them to be specific and complete. Is this the real problem or is there a more fundamental problem?

Step 3: Identification of Alternatives.

What are some options for dealing with the problem? Ask the patient to list them on paper so they can readily see them. Try to have the group come up with a list of at least five possible solutions. This will give them more of a chance of choosing the best solution and give them some alternatives if their first choice doesn't work.

Step 4: Projected Consequences of Each Alternative.

What are the probable outcomes of each option? Have the group ask the person the following questions:

- What is the best possible thing that could happen if you choose this alternative?
- What is the worst possible thing that could happen?
- What is the most likely thing that will happen?
- What is your reaction (thoughts, feelings, memories, and future projections) when you think about implementing that alternative?

Step 5: Decision.

Have the group ask the person which option offers the best outcomes and seems to have the best chance for success. Ask them to make a decision based upon the alternatives they have.

Step 6: Action.

Once they have decided on a solution to the problem, they need to plan how they will carry it out. Making a plan answers the question, "What are you going to do about it?" A plan is a road map to achieve a goal. There are long range goals and short range goals. Long range goals are achieved along with short range goals. One step at a time.

Step 7: Followup.

Ask the person to carry out his or her plan and report on how it is working.

Most problems will not be solved by presenting them one time in group session. Personal problem solving is a process that requires time. It may require three to six presentations of a problem, accompanied by specific assignments completed between group sessions to bring a problem to full resolution. Patients should be given a limited time to present a problem or the summary of an assignment. As a general rule, patients should not work in group for more than 20 minutes.

Not every person will work on a problem during each session. There is an 80/20 rule for group treatment. Eighty percent of the benefit of group treatment occurs from learning how to become responsibly involved in helping others to solve their problems. Only 20 percent of the benefit is derived from working on personal problem issues.

Feedback

When you reach a point where part of the problem solving process is completed or an assignment is presented, group members and the counselor should give feedback. The counselor should go last. Feedback should be given by having the members complete the following:

- My gut level reactions is . . . (A feeling, thought, or how members can identify with the patient who presented)
- I think that how this affects your recovery is . . .
- What I think of you as a person is . . .

The purpose of this feedback exercise is to practice communication skills, learn to give and take feedback, and use the group for problem solving. The counselor may give an assignment to the patient if it would be helpful to continue to learn more about how to solve this issue.

The Closure Exercise (15 minutes)

When there is approximately 15 minutes left in the group session, the counselor will ask the members the following:

- What is the most important thing you learned in group this evening? It is important to write this down in your notebook.
- What are you going to change about your behavior? Write this down in your notebook.
- Share with the group what you learned and what changes you are willing to make.

Each participant will then briefly review his or her answers to those questions with the group. The counselor then adjourns the group.

The Debriefing Session

The debriefing session is designed to review the patient's problems and progress, prevent counselor burnout, and improve the group skills of the counselor. If this can be done with other counselors running similar groups, it is especially helpful. A brief review of each patient is completed, outstanding group members and events are identified, progress and problems are discussed, and the personal feelings and reactions of the counselor are reviewed.

Outline for Group Counseling Sessions

Opening Procedure—Format (5 minutes)

- Form a tight circle.
- Do physical and eye contact exercises.
- Do centering (breathing) exercise.
- Take attendance to identify moods.

Opening Procedure—Purpose

- Establish control.
- Get group members in contact with one another.
- Get group focused.
- Check members' attitude and mood.

Reactions to Last Sessions—Format (15 minutes)

- Ask what patients thought about during last session.
- Ask how they feel during last session.
- Ask which three people stood out from last session and why.

Reactions to Last Session—Purpose

- Communication training.
- Memory training.
- Tie together group experience.
- Force interest in other group members.
- Initiate high quality group interaction.
- Test motivation.
- Create opportunity for "no fault" communication.

Report on Assignments—Format (10 minutes)

- Find out who had an assignment.
- Ask whether they completed it.
- If yes, ask what they learned.
- If no, ask what happened when they tried to complete it.
- Ask how important is it for them to present this in group tonight. (Rate 1-10.)
- Discuss any other problems that need to be worked on in group. (Rate 1-10.)

Report on Assignments—Purpose

- Accountability (getting only what you expect and inspect).
- Continuity (ensuring that all assignments are completed).

Setting the Agenda—Format (3 minutes)

- DECIDE and announce: The order of presentation by the group members.

Setting the Agenda—Purpose

- To identify the members who need to work in group.
- To review a brief description of the issue the member wants to work on.
- To establish priorities based on:
 - Problem severity
 - History of participation

Problem Solving Process—Format (70 minutes)

- Problem presentation.
- Questioning by the group.
- Feedback from group members.
- Feedback from the group counselor, if appropriate.
- Closure by the therapist.

Problem Solving Process—Purpose

- To present issues.
- To clarify the issue through questioning by the group.
- To receive feedback from group members.
- To develop assignments for continued progress.

Presenting a Problem in Group

- "The problem I want to work on is . . ."
- "This first became a problem when . . ."
- "The relationship of this problem to my addiction is . . ."
- "I have tried to solve this problem in the past by . . ."

Goals of Group Questioning

- To establish rapport by active listening.
- To encourage group members to know and understand the member who is working on a problem.
- To convey the message, "You are listened to, understood, taken seriously, and affirmed as a person."

Types of Questions

- Open—Cannot be answered with a "yes" or "no."
- Focus—Forces a choice between limited options.
- Closed—Forces a "yes" or "no" answer.
- Leading—Forces consideration of a new point of view.

The "EIAG" Method of Questioning

- **E**—EXPERIENCE: "What exactly did you experience and why is it a problem?"
- **I**—IDENTIFICATION: "Can you identify what the important parts, elements, or outcomes of the experience were for you?"
- **A**—ANALYZE: "Why was this experience important? What is its meaning or significance?"
- **G**—GENERALIZE: "What did you learn from this experience and how will you apply what you learned to other experiences?"

Addiction-Focused Questions

- How did this problem or experience contribute to the development of your addiction?
- How did this problem or experience affect your willingness or ability to recognize or seek treatment for your addiction?
- How did this problem or experience affect your willingness or ability to stay sober or maintain your recovery program?
- How did this problem or experience set you up to relapse in the future?

Giving Feedback in Group

- "My gut level reaction to your problem or assignment is . . ."
- "I believe your problem is . . ."
- "How I feel about you as a person is . . ."

The Timing of Change

- No problem is ever solved in one group presentation.
- To solve a single problem requires three to six group presentations.
- Each problem will need to be broken down into pieces that can be worked on in 20- to 30-minute sessions.
- Limit each presentation to 20–30 minutes.
- Allow time for two to four patients to work in each group.

The Problem Solving Process

- Problem identification
- Problem clarification
- Identification of alternatives
- Projecting the consequences of each alternative (best, worst, most likely)
- Decision
- Action
- Followup

The Closure Exercise—Format (15 minutes)

- Write down the most important thing you learned in group today.
- Write down what you will do differently as a result of what you learned.
- Explain to the group the most important thing you learned in group and what you will do differently as a result.

The Closure Exercise—Purpose

- To ensure that each group member understands and integrates the group experience.
- To assist in documenting the group process.

Adjournment

- Ask group members to report if they are not going to be in the next group session.
- Confirm the day, date, and time of the next group.
- The group is officially ended.

The Debriefing Session—Format

- Patient review: Review the progress and problems of each patient.
- Outstanding group members: Think about and record which group members stood out the most in today's group and why.
- Outstanding events: Think about and record any outstanding positive or negative events in the group.
- Problems—Progress: Think about and record any problems or progress observed in the overall management of the group.
- Personal feelings and reactions: Think about and record any personal feelings and reactions about the group.

The Debriefing Session—Purpose

- To review patient progress and problems.
- To prevent counselor burnout.
- To train and develop the skills of the counselor team.
- Debriefing is critical to long-term group success.

Contributors and Attributions

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CHAPTER OVERVIEW

3: Untitled Chapter 3

3.1: Facilitating Discussions about Intercultural Communication Issues

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3.1: Facilitating Discussions about Intercultural Communication Issues

Perhaps you may have noticed the theme of inequality as we have discussed topics like “unequal access to resources and benefits,” racial discrimination, and racism. You may have also thought, “oh, my, this is going to be a touchy chapter to read and discuss in class” or “this is interesting and relevant, but I feel uncomfortable talking about this as I don’t want to offend anyone.” These are very common and understandable reactions and ones we hear when we teach this subject matter. Hopefully, your instructor has set up a safe, open, and respectful classroom environment to facilitate such discussions. The fact that you are self-reflective of your feelings and how to express them to others is a great start! We too want you to be able to discuss this material both in and out of your class in a productive and self-reflective manner. To facilitate that goal we have included some additional concepts— privilege, ethnocentrism, whiteness, and political correctness—that are useful when considering your own cultural identity, your place in society, and your communication with others.

As it was so eloquently put by Carey W. James, “I haven’t given up the quest, typically if idealistically American, for an open, nonascriptive basis of community life: one in which neighbors help one another out--you know, lend out the lawnmower, come to the funeral, take part in the town meeting-but do not ask one another too many questions about their private lives and pretty much ignore the color of skin, the shapes of noses and eyes, or the distribution of X/Y chromosomes.” This scholar is one of many people in the fight for political correctness to prevail.

Privilege

Hopefully, you have been thinking about your own cultural identity as you have been reading this chapter. If so, then you have been thinking about labels that define you culturally. Maybe you have defined yourself as female, Latina, and heterosexual. Or maybe you have labeled yourself as gay, white, working-class, and male. When we give ourselves labels such as these, often we ask ourselves, “Where do I fit in?” This is a good question to ask and demonstrates a recognition of the fact that you belong to more than one culture and that your cultures intersect in various ways. The most significant manifestation of these intersections is **power**—**the ability to influence others and control our lives**. From the statistics given earlier in the chapter and from your own experiences, you should realize that some groups have more power than others. These people are what we refer to as the **dominant group: white, male, Christian, middle-class, able-bodied, educated, and heterosexual**. People whose cultural identities do not conform to this model are the **nondominant groups** and **have less sociopolitical and economic power**.

Peggy McIntosh uses the term **privilege** to refer to **the power of dominant groups**. She defines privilege as an invisible knapsack of advantages that some people carry around. They are invisible because they are often not recognized, seen as normative (i.e., “that’s just the way things are”), seen as universal (i.e., “everyone has them”), or used unconsciously. Below is a list of some of the privileges McIntosh identifies. Can you think of others?

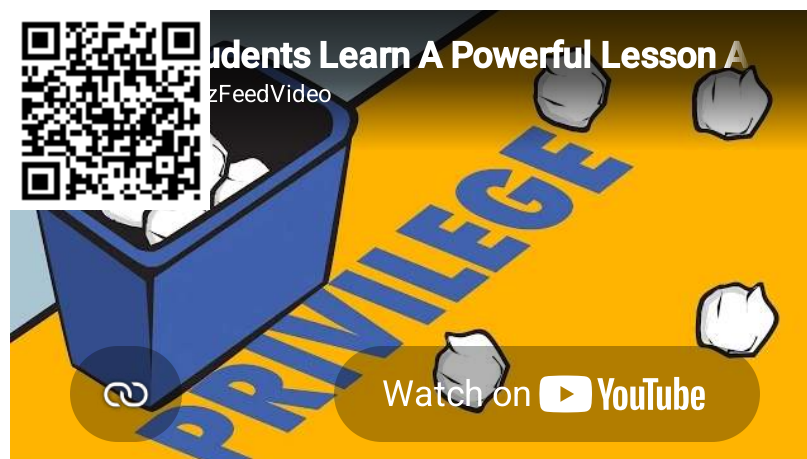
1. I can, if I wish, arrange to be in the company of people of my race most of the time.
2. If I should need to move, I can be pretty sure of renting or purchasing housing in an area that I can afford and in which I would want to live.
3. I can be pretty sure that my neighbors in such a location will be neutral or pleasant to me.
4. I can go shopping alone most of the time, pretty well assured that I will not be followed or harassed.
5. I can turn on the television or open to the front page of the paper and see people of my race widely represented.
6. When I am told about our national heritage or about “civilization,” I am shown that people of my color made it what it is.
7. I can be sure that my children will be given curricular materials that testify to the existence of their race.
8. If I want to, I can be pretty sure of finding a publisher for this piece on white privilege.
9. I can go into a music shop and count on finding the music of my race represented, into a supermarket and find the staple foods that fit with my cultural traditions, into a hairdresser’s shop and find someone who can deal with my hair.
10. Whether I use checks, credit cards, or cash, I can count on my skin color not to work against the appearance of financial reliability.
11. I can arrange to protect my children most of the time from people who might not like them.
12. I can swear, or dress in second-hand clothes, or not answer letters without having people attribute these choices to the bad morals, the poverty, or the illiteracy of my race.
13. I can speak in public to a powerful male group without putting my race on trial.
14. I can do well in a challenging situation without being called a credit to my race.
15. I am never asked to speak for all the people of my racial group.

16. I can remain oblivious of the language and customs of persons of color, who constitute the world's majority, without feeling in my culture any penalty for such oblivion.
17. I can criticize our government and talk about how much I fear its policies and behavior without being seen as a cultural outsider.
18. I can be pretty sure that if I ask to talk to "the person in charge" I will be facing a person of my race.
19. If a traffic cop pulls me over, or if the IRS audits my tax return, I can be sure I haven't been singled out because of my race.
20. I can easily buy posters, postcards, picture books, greeting cards, dolls, toys, and children's magazines featuring people of my race.
21. I can go home from most meetings of organizations I belong to feeling somewhat tied in rather than isolated, out of place, outnumbered, unheard, held at a distance, or feared.
22. I can take a job with an affirmative action employer without having coworkers on the job suspect that I got it because of race.
23. I can choose public accommodation without fearing that people of my race cannot get in or will be mistreated in the places I have chosen.
24. I can be sure that if I need legal or medical help my race will not work against me.
25. If my day, week, or year is going badly, I need not ask of each negative episode or situation whether it has racial overtones.
26. I can choose blemish cover or bandages in "flesh" color that more or less match my skin.

McIntosh admits, "My perception is that colleges and universities are the main institutions that are raising awareness of the relationship between privilege and oppression, but that this awareness is needed throughout all public and private sectors of the United States; the ability to see privilege should be in the minds of all citizens" (195). As you think about privilege and the resulting advantages that some groups have over others, you should also keep in mind two facts. One, privilege is a relative concept that varies according to context. In some situations we may be more privileged than others, and in order to access some of that privilege one may decide to highlight or conceal parts of their identity. For example, unless a person tells you, you have no way of knowing their sexual orientation. Thus, a gay man might decide to "pass" as straight at a family reunion to avoid conflict from a heterosexual family. The fact that he can choose to pass as a straight man is also a form of privilege. A black woman cannot decide to pass as a white woman, for example. Two, we may have aspects of our identities that are simultaneously advantaged and disadvantaged. The gay, white, working-class, male above is advantaged by the fact that he has light skin and is male, and is disadvantaged by the fact that he is gay and working-class.

Another example is from Nathan Pyle, a BuzzFeed staff member who wrote an article entitled, [10 Privileges I Have Complained About](#). Read the article and think about privileges you have complained about. Pyle states, "becoming self-aware of my privilege is a journey I'm still on." Perhaps we should all start becoming self-aware of our privileges in America.

Watch this video, [Students Learn A Powerful Lesson About Privilege](#)



Ethnocentrism

One of the first steps to communicating sensitively and productively about cultural identity is to be able to name and recognize one's identity and the relative privilege that it affords. Similarly important, is a recognition that one's cultural standpoint is not everyone's standpoint. Our views of the world, what we consider right and wrong, normal or weird, are largely influenced by our cultural position or standpoint: the intersections of all aspects of our identity. One common mistake that people from all cultures are guilty of is **ethnocentrism**—*placing one's own culture and the corresponding beliefs, values, and behaviors in the center*. When

we do this we view our position as normal and right, and evaluate all other cultural systems against our own. If you want to learn more about ethnocentrism, [view this slide show](#).

Ethnocentrism shows up in small and large ways: the WWII Nazi's elevation of the Aryan race and the corresponding killing of Jews, Gypsies, gays and lesbians, and other non-Aryan groups is one of the most horrific ethnocentric acts in history. However, ethnocentrism shows up in small and seemingly unconscious ways as well. In American culture, if you decided to serve dog meat as an appetizers at your cocktail party you would probably disgust your guests and the police might even arrest you because the consumption of dog meat is not culturally acceptable. However, in China "it is neither rare nor unusual" to consume dog meat (Wingfield-Hayes). In the Czech Republic, the traditional Christmas dinner is carp and potato salad. Imagine how your U.S. family might react if you told them you were serving carp and potato salad for Christmas. In the Czech Republic, it is a beautiful tradition, but in America, it might not receive a warm welcome. Our cultural background influences every aspect of our lives from the food we consume to classroom curriculum.

Ethnocentrism may show up in Literature classes, for example, as cultural bias dictates which "great works" students are going to read and study. More often than not, these works represent the given culture (i.e., reading French authors in France and Korean authors in Korea). This ethnocentric bias has received some challenge in United States' schools, as teachers make efforts to create a multicultural classroom by incorporating books, short stories, and traditions from non-dominant groups. In the field of geography there has been an ongoing debate about the use of a Mercator map versus a Peter's Projection map. The arguments reveal cultural biases toward the Northern, industrialized nations. To see this bias, [follow this link](#).

Case In Point

The Greenland Problem

The Mercator projection creates increasing distortions of size as you move away from the equator. As you get closer to the poles the distortion becomes severe. Cartographers refer to the inability to compare size on a Mercator projection as "the Greenland Problem." Greenland appears to be the same size as Africa, yet Africa's land mass is actually fourteen times larger. Because the Mercator distorts size so much at the poles it is common to crop Antarctica off the map. This practice results in the Northern Hemisphere appearing much larger than it really is. Typically, the cropping technique results in a map showing the equator about 60% of the way down the map, diminishing the size and importance of the developing countries.

Greenland is 0.8 million sq. miles and Africa is 11.6 million sq. miles, yet the often look roughly the same size on maps.

This was convenient, psychologically and practically, through the eras of colonial domination when most of the world powers were European. It suited them to maintain an image of the world with Europe at the center and looking much larger than it really was. Was this conscious or deliberate? Probably not, as most map users probably never realized the Eurocentric bias inherent in their world view. When there are so many other projections to choose from, why is it that today the Mercator projection is still such a widely recognized image used to represent the globe? The answer may be simply convention or habit. The inertia of habit is a powerful force.

Whiteness

If you are White, how would you describe your culture? When we ask this question to our students we find that White students are often uncomfortable with the question, feel guilty about self-identifying as White, or claim that White people do not have a culture. Gordon Alley-Young says, "The invisibility of whiteness and white privilege for many people is what makes it difficult to name and thus to disrupt" (312). These sentiments have lead an increasing amount of scholars in a variety of disciplines such as Sociology, Women's Studies, Anthropology, English, as well as Communication to study the concept of Whiteness. Orbe and Harris explain why exploring this concept is important by explaining that "[i]t helps us all view communication as a racialized process [which] sharpens our awareness of how racial categorization is used to reinforce old hierarchies in which some races are more superior than others [and that] whiteness studies also assign each person a role in race relations" (89).

view communication as a racialized process—meaning that our communication is structured by larger societal and racial dynamics. Second, understanding Whiteness sharpens our awareness of how racial categorization is used to reinforce old hierarchies in which some races are more superior than others. This helps us recognize how Whiteness can be used to signify dominance, privilege, and advantage in the United States. And, third, through studying and recognizing the effects of Whiteness,

each person plays a role in race relations. White people can no longer sit on the sidelines and claim “it’s a black problem” when discussing interracial conflict. (82-83)

Overall, it removes the White race from the often-unidentified “normative” group and provides a context for studying, talking about, and hopefully improving race relations.

The above discussion about privilege and Whiteness is not meant to suggest that those people with sociopolitical privilege should feel ashamed or guilty. This is often a trap that people fall into and it can shut down important thinking and conversations about intercultural communication. We want everyone to realize that they have a racial identity and thus are an important part of improving race relations. Race relations is not just a subject that concerns minorities—it concerns everyone as we all play a part and benefit whether consciously or unconsciously.

Political Correctness

Case In Point

Since his first kneel, Colin Kaepernick has faced backlash from the NFL, fans, “patriots,” and even Donald Trump. He has been told that he will never play in the NFL again and fans have taken to burning his jersey. However, taking a knee has also become a large protest for the unfair treatment of African Americans by police. A peaceful nonverbal communication that says more than most can comprehend.

Another claim or label that may be used to discount such difficult discussions is Political Correctness, or “PC” as it has been dubbed in the popular press. Opponents of multiculturalism and diversity studies try and dismiss such topics as “that’s just PC.” Luckily, some of the heated debate about PC have quieted in recent years but the history lingers. In short, political correctness refers to “the elimination of speech that often works to exclude, oppress, demean, or harass certain groups” (Orbe and Harris 58, Remar). The debate largely focused around competing interpretations of the First Amendment right to free speech and the Fourteenth Amendment’s right to equal access to education. No matter what your position on this issue, we want to simply recognize two facts. One, that much of the PC debate and fury was largely misrepresented and hyped in the mainstream media by the use of extreme examples and a slippery-slope argument. Rush Limbaugh, for example, became famous for claiming that an awareness and sensitivity of language choice would lead to the “thought Police” or “PC police.” Two, that words and labels have great power to create perceptions, realities and identities. Toward that aim, we will discuss the power of language in greater detail in the following section.

Contributions and Affiliations

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CHAPTER OVERVIEW

4: Untitled Chapter 4

4.1: Trauma Awareness

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4.1: Trauma Awareness

In This Chapter

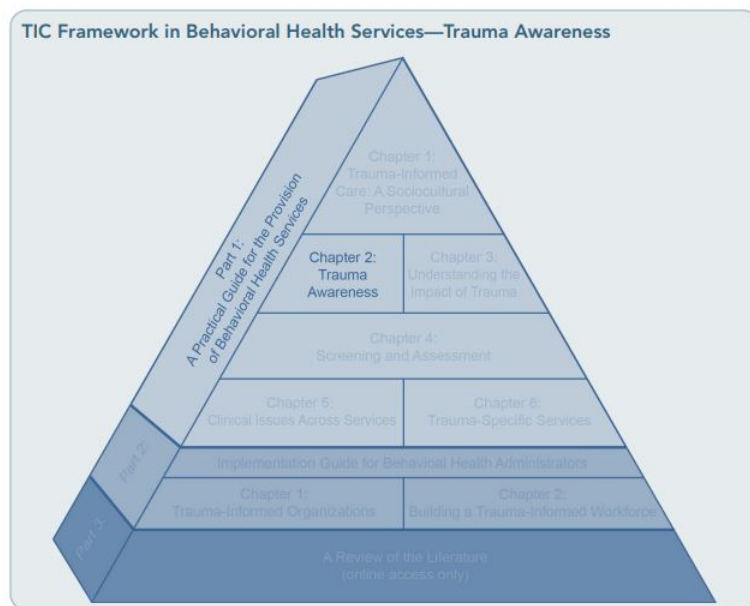
- Types of Trauma
- Characteristics of Trauma
- Individual and Sociocultural Features

Traumatic experiences typically do not result in long-term impairment for most individuals. It is normal to experience such events across the lifespan; often, individuals, families, and communities respond to them with resilience. This chapter explores several main elements that influence why people respond differently to trauma. Using the social-ecological model outlined in Part 1, Chapter 1, this chapter explores some of the contextual and systemic dynamics that influence individual and community perceptions of trauma and its impact. The three main foci are: types of trauma, objective and subjective characteristics of trauma, and individual and sociocultural features that serve as risk or protective factors.

This chapter's main objective is to highlight the key characteristics of traumatic experiences. Trauma-informed behavioral health service providers understand that many influences shape the effects of trauma among individuals and communities—it is not just the event that determines the outcome, but also the event's context and the resultant interactions across systems.

Types of Trauma

The following section reviews various forms and types of trauma. It does not cover every conceivable trauma that an individual, group, or community may encounter. Specific traumas are reviewed only once, even when they could fit in multiple categories of trauma. Additionally, the order of appearance does not denote a specific trauma's importance or prevalence, and there is no lack of relevance implied if a given trauma is not specifically addressed in this Treatment Improvement Protocol (TIP). The intent is to give a broad perspective of the various categories and types of trauma to behavioral health workers who wish to be trauma informed.



TIC Framework in Behavioral Health Services – Trauma Awareness

Natural or Human-Caused Traumas

The classification of a trauma as natural or caused by humans can have a significant impact on the ways people react to it and on the types of assistance mobilized in its aftermath (see Exhibit 1.2-1 for trauma examples). Natural traumatic experiences can directly affect a small number of people, such as a tree falling on a car during a rainstorm, or many people and communities, as with a hurricane. Natural events, often referred to as “acts of God,” are typically unavoidable. Human-caused traumas are caused by human failure (e.g., technological catastrophes, accidents, malevolence) or by human design (e.g., war). Although multiple

factors contribute to the severity of a natural or human-caused trauma, traumas perceived as intentionally harmful often make the event more traumatic for people and communities.

For information on resources to prepare States, Territories, and local entities to deliver effective mental health and substance abuse responses during disasters, contact the Substance Abuse and Mental Health Services Administration's (SAMHSA's) Disaster Technical Assistance Center:

4350 East -West Hwy, Suite 1100

Bethesda, MD 20814 -6233

Phone: 1 -800 -308 -3515

Fax: 1 -800 -311 -7691

Email: DTAC@samhsa.hhs.gov

How survivors of natural trauma respond to the experience often depends on the degree of devastation, the extent of individual and community losses, and the amount of time it takes to reestablish daily routines, activities, and services (e.g., returning to school or work, being able to do laundry, having products to buy in a local store). The amount, accessibility, and duration of relief services can significantly influence the duration of traumatic stress reactions as well as the recovery process.

Alongside the disruption of daily routines, the presence of community members or outsiders in affected areas may add significant stress or create traumatic experiences in and of themselves. Examples include the threat of others stealing what remains of personal property, restrictions on travel or access to property or living quarters, disruption of privacy within shelters, media attention, and subsequent exposure to repetitive images reflecting the devastation. Therefore, it isn't just the natural disaster or event that can challenge an individual or community; often, the consequences of the event and behavioral responses from others within and outside the community play a role in pushing survivors away from effective coping or toward resilience and recovery.

Human-caused traumas are fundamentally different from natural disasters. They are either intentional, such as a convenience store robbery at gunpoint, or unintentional, such as the technological accident of a bridge collapse (as occurred in Minneapolis, Minnesota, in 2007; U.S. Fire Administration, 2007). The subsequent reactions to these traumas often depend on their intentionality. However, a person or group of people is typically the target of the survivors' anger and blame. Survivors of an unintentionally human-caused traumatic event may feel angry and frustrated because of the lack of protection or care offered by the responsible party or government, particularly if there has been a perceived act of omission. After intentional human-caused acts, survivors often struggle to understand the motives for performing the act, the calculated or random nature of the act, and the psychological makeup of the perpetrator(s).

Exhibit 1.2-1 Trauma Examples

Caused Naturally	Caused by People	
	Accidents, Technological Catastrophes	Intentional Acts
Tornado Lightning strike Wildfire Avalanche Physical ailment or disease Fallen tree Earthquake Dust storm Volcanic eruption Blizzard Hurricane Cyclone Typhoon Meteorite Flood Tsunami Epidemic Famine Landslide or fallen boulder	Train derailment Roofing fall Structural collapse Mountaineering accident Aircraft crash Car accident due to malfunction Mine collapse or fire Radiation leak Crane collapse Gas explosion Electrocution Machinery-related accident Oil spill Maritime accident Accidental gun shooting Sports-related death	Arson Terrorism Sexual assault and abuse Homicides or suicides Mob violence or rioting Physical abuse and neglect Stabbing or shooting Warfare Domestic violence Poisoned water supply Human trafficking School violence Torture Home invasion Bank robbery Genocide Medical or food tampering

Case Illustrations: Quecreek Mine Flood and Greensburg's Tornado

Quecreek Mine Flood

The year following the rescue of nine miners from the Quecreek mine in western Pennsylvania in 2002 was a difficult one for residents of Somerset County. The dazzle of publicity surrounding a handful of workers from a small town, tension between miners and rescuers, and animosity over money for movie and book deals, in addition to the trauma itself, resulted in a rescuer's suicide, a number of miners having trauma-related symptoms, and several rescuers needing to seek treatment for posttraumatic stress disorder (PTSD; Goodell, 2003).

Greensburg's Tornado

Greensburg, a small town in southern Kansas, was hit by a large tornado in 2007 that killed 11 residents and leveled 95 percent of the town while causing severe damage to the remaining 5 percent. Families and community members experienced significant grief and traumatic stress after the disaster. Yet today, Greensburg is rebuilding with a focus on being “green”—that is, environmentally responsible—from design to construction and all the way through demolition. This town has the highest number of Leadership in Energy and Environmental Design–certified buildings in the world. A reality television show about the town's reinvention ran for three seasons, demonstrating the town's residents and business owners working with local government and various corporations to make their home an even better place than it was before the tornado.

Individual, Group, Community, and Mass Traumas

In recognizing the role of trauma and understanding responses to it, consider whether the trauma primarily affected an individual and perhaps his or her family (e.g., automobile accident, sexual or physical assault, severe illness); occurred within the context of a group (e.g., trauma experienced by first responders or those who have seen military combat) or community (e.g., gang-related shootings); transpired within a certain culture; or was a large-scale disaster (e.g., hurricane, terrorist attack). This context can have significant implications for whether (and how) people experience shame as a result of the trauma, the kinds of support and compassion they receive, whether their experiences are normalized or diminished by others, and even the kinds of services they are offered to help them recover and cope.

Individual trauma

An individual trauma refers to an event that only occurs to one person. It can be a single event (e.g., mugging, rape, physical attack, work-related physical injury) or multiple or prolonged events (e.g., a life-threatening illness, multiple sexual assaults). Although the trauma directly affects just one individual, others who know the person and/or are aware of the trauma will likely experience emotional repercussions from the event(s) as well, such as recounting what they said to the person before the event, reacting in disbelief, or thinking that it could just as easily have happened to them, too.

Survivors of individual trauma may not receive the environmental support and concern that members of collectively traumatized groups and communities receive. They are less likely to reveal their traumas or to receive validation of their experiences. Often, shame distorts their perception of responsibility for the trauma. Some survivors of individual traumas, especially those who have kept the trauma secret, may not receive needed comfort and acceptance from others; they are also more likely to struggle with issues of causation (e.g., a young woman may feel unduly responsible for a sexual assault), to feel isolated by the trauma, and to experience repeated trauma that makes them feel victimized.

Advice to Counselors: Working With Clients Who Have Experienced Individual Traumas

In working with clients who have histories of individual trauma, counselors should consider that:

- Empathy, or putting oneself in the shoes of another, is more potent than sympathy (expressing a feeling of sorrow for another person).
- Some clients need to briefly describe the trauma(s) they have experienced, particularly in the early stages of recovery. Strategies that focus on reexperiencing the trauma, retrieving feelings related to the trauma, and bringing past experiences to the forefront should only be implemented if trauma-specific treatment planning and services are available.
- Understanding the trauma, especially in early recovery, should begin with educating the client about and normalizing trauma-related symptoms, creating a sense of safety within the treatment environment, and addressing how trauma symptoms may interfere with the client's life in the present.

- It is helpful to examine how the trauma affects opportunities to receive substance abuse and/or mental health treatment as well as treatment for and recovery from the trauma itself (e.g., by limiting one's willingness to share in or participate in group counseling).
- Identifying and exploring strengths in the client's history can help the client apply those strengths to his or her ability to function in the present.

Physical injuries

Physical injuries are among the most prevalent individual traumas. Millions of emergency room (ER) visits each year relate directly to physical injuries. Most trauma patients are relatively young; about 70 percent of injury-related ER cases are people younger than 45 years old (McCaig & Burt, 2005). Dedicated ER hospital units, known as "trauma centers," specialize in physical traumas such as gunshot wounds, stabbings, and other immediate physical injuries. The term "trauma" in relation to ERs does not refer to psychological trauma, which is the focus of this TIP, yet physical injuries can be associated with psychological trauma. Sudden, unexpected, adverse health-related events can lead to extensive psychological trauma for patients and their families.

Excessive alcohol use is the leading risk factor for physical injuries; it's also the most promising target for injury prevention. Studies consistently connect injuries and substance use (Gentilello, Ebel, Wickizer, Salkever, & Rivara, 2005); nearly 50 percent of patients admitted to trauma centers have injuries attributable to alcohol abuse and dependence (Gentilello et al., 1999). One study found that two thirds of ambulatory assault victims presenting to an ER had positive substance use urinalysis results; more than half of all victims had PTSD 3 months later (Roy-Byrne et al., 2004). Nearly 28 percent of patients whose drinking was identified as problematic during an ER visit for a physical injury will have a new injury within 1 year (Gentilello et al., 2005). For further information, see TIP 16, Alcohol and Other Drug Screening of Hospitalized Trauma Patients (Center for Substance Abuse Treatment [CSAT], 1995a).

Acute stress disorder (ASD) prevalence among patients at medical trauma centers is very high, making trauma-related disorders some of the most common complications seen in physically injured patients. Clients who have sustained serious injuries in car crashes, fires, stabbings, shootings, falls, and other events have an increased likelihood of developing trauma-related mental disorders. Research suggests that PTSD and/or problem drinking is evident in nearly 50 percent of patients 1 year after discharge from trauma surgical units. (Zatzick, Jurkovich, Gentilello, Wisner, & Rivara, 2002)

Group trauma

The term "group trauma" refers to traumatic experiences that affect a particular group of people. This TIP intentionally distinguishes group trauma from mass trauma to highlight the unique experiences and characteristics of trauma-related reactions among small groups. These groups often share a common identity and history, as well as similar activities and concerns. They include vocational groups who specialize in managing traumas or who routinely place themselves in harm's way—for example, first responders, a group including police and emergency medical personnel. Some examples of group trauma include crews and their families who lose members from a commercial fishing accident, a gang whose members experience multiple deaths and injuries, teams of firefighters who lose members in a roof collapse, responders who attempt to save flood victims, and military service members in a specific theater of operation.

Survivors of group trauma can have different experiences and responses than survivors of individual or mass traumas. Survivors of group trauma, such as military service members and first responders, are likely to experience repeated trauma. They tend to keep the trauma experiences within the group, feeling that others outside the group will not understand; group outsiders are generally viewed as intruders. Members may encourage others in the group to shut down emotionally and repress their traumatic experiences—and there are some occupational roles that necessitate the repression of reactions to complete a mission or to be attentive to the needs at hand.

Group members may not want to seek help and may discourage others from doing so out of fear that it may shame the entire group. In this environment, members may see it as a violation of group confidentiality when a member seeks assistance outside the group, such as by going to a counselor.

Group members who have had traumatic experiences in the past may not actively support traumatized colleagues for fear that acknowledging the trauma will increase the risk of repressed trauma-related emotions surfacing. However, groups with adequate resources for helping group members can develop a stronger and more supportive environment for handling subsequent traumas. These main group features influence the course of shortand long-term adjustments, including the development of traumatic stress symptoms associated with mental and substance use disorders.

Certain occupational groups are at greater risk of experiencing trauma—particularly multiple traumas. This TIP briefly reviews two main groups as examples in the following sections: first responders and military service members. For more detailed information on the impact of trauma and deployment, refer to the planned TIP, *Reintegration-Related Behavioral Health Issues in Veterans and Military Families* (SAMHSA, planned f).

First responders

First responders are usually emergency medical technicians, disaster management personnel, police officers, rescue workers, medical and behavioral health professionals, journalists, and volunteers from various backgrounds. They also include lifeguards, military personnel, and clergy. Stressors associated with the kinds of traumatic events and/or disasters first responders are likely to experience include exposure to toxic agents, feeling responsible for the lives of others, witnessing catastrophic devastation, potential exposure to gruesome images, observing human and animal suffering and/or death, working beyond physical exhaustion, and the external and internal pressure of working against the clock.

Military service members

Military personnel are likely to experience numerous stressors associated with trauma. Service members who have repeatedly deployed to a war zone are at a greater risk for traumatic stress reactions (also known as combat stress reaction or traumatic stress injury), other military personnel who provide support services are also at risk for traumatic stress and secondary trauma (refer to the glossary portion of the “How This TIP Is Organized” section that precedes Part 1, Chapter 1, of this TIP). So too, service members who anticipate deployment or redeployment may exhibit psychological symptoms associated with traumatic stress. Some stressors that military service members may encounter include working while physically exhausted, exposure to gunfire, seeing or knowing someone who has been injured or killed, traveling in areas known for roadside bombs and rockets, extended hypervigilance, fear of being struck by an improvised explosive device, and so forth.

Trauma affecting communities and cultures

Trauma that affects communities and cultures covers a broad range of violence and atrocities that erode the sense of safety within a given community, including neighborhoods, schools, towns, and reservations. It may involve violence in the form of physical or sexual assaults, hate crimes, robberies, workplace or gang-related violence, threats, shootings, or stabbings—for example, the school shooting at Virginia Polytechnic Institute and State University in 2007. It also includes actions that attempt to dismantle systemic cultural practices, resources, and identities, such as making boarding school attendance mandatory for Native American children or placing them in non-Native foster homes. Cultural and/or community-based trauma can also occur via indifference or limited responsiveness to specific communities or cultures that are facing a potential catastrophe. Cultural traumas are events that, whether intentionally or not, erode the heritage of a culture—as with prejudice, disenfranchisement, and health inequities (e.g., late prenatal care, inability to afford medications, limited access to culturally appropriate health education, vicinity and quality of affordable medical services), among other examples.

“The excitement of the season had just begun, and then, we heard the news, oil in the water, lots of oil killing lots of water. It is too shocking to understand. Never in the millennium of our tradition have we thought it possible for the water to die, but it is true.” — Chief Walter Meganack, Port Graham, 1989

Of all the groups negatively affected by the Exxon Valdez oil spill, in many ways Alaska Natives were the most devastated. The oil spill destroyed more than economic resources; it shook the core cultural foundation of Native life. Alaska Native subsistence culture is based on an intimate relationship with the environment. Not only does the environment have sacred qualities for Alaska Natives; their survival also depends on the well-being of the ecosystem and the maintenance of cultural norms of subsistence. The spill directly threatened the well-being of the environment, disrupted subsistence behavior, and severely disturbed the sociocultural milieu of Alaska Natives.

Source: Gill & Picou, 1997, pp. 167–168.

Historical trauma

Historical trauma, known also as generational trauma, refers to events that are so widespread as to affect an entire culture; such events also have effects intense enough to influence generations of the culture beyond those who experienced them directly. The enslavement, torture, and lynching of African Americans; the forced assimilation and relocation of American Indians onto reservations; the extermination of millions of Jews and others in Europe during World War II; and the genocidal policies of the Hutus in Rwanda and the Khmer Rouge in Cambodia are examples of historical trauma.

In the past 50 years, research has explored the generational effects of the Holocaust upon survivors and their families. More recent literature has extended the concept of historical or generational trauma to the traumatic experiences of Native Americans. Reduced population, forced relocation, and acculturation are some examples of traumatic experiences that Native people have endured across centuries, beginning with the first European presence in the Americas. These tragic experiences have led to significant loss of cultural identity across generations and have had a significant impact on the well-being of Native communities (Whitbeck, Chen, Hoyt, & Adams, 2004). Data are limited on the association of mental and substance use disorders with historical trauma among Native people, but literature suggests that historical trauma has repercussions across generations, such as depression, grief, traumatic stress, domestic violence, and substance abuse, as well as significant loss of cultural knowledge, language, and identity (Gone, 2009). Historical trauma can increase the vulnerability of multiple generations to the effects of traumas that occur in their own lifetimes.

Mass trauma

Mass traumas or disasters affect large numbers of people either directly or indirectly. It is beyond the scope of this TIP to cover any specific disaster in detail; note, however, that mass traumas include large-scale natural and human-caused disasters (including intentional acts and accidents alike). Mass traumas may involve significant loss of property and lives as well as the widespread disruption of normal routines and services. Responding to such traumas often requires immediate and extensive resources that typically exceed the capacity of the affected communities, States, or countries in which they occur. Recent examples of such large-scale catastrophes include:

- In January 2010, a massive earthquake hit Haiti, killing hundreds of thousands of people and leaving over a million homeless.
- A nuclear reactor meltdown in the Ukraine in 1986 resulted in a technological and environmental disaster that affected tens of millions of people.
- The tsunami in the Indian Ocean in 2005 left hundreds of thousands dead in nine countries.

One factor that influences an individual's response to trauma is his or her ability to process one trauma before another trauma occurs. In mass traumas, the initial event causes considerable destruction, the consequences of which may spawn additional traumas and other stressful events that lead to more difficulties and greater need for adjustments among survivors, first responders, and disaster relief agencies. Often, a chain reaction occurs. Take, for example, Hurricane Katrina and its impact on the people of Louisiana and other coastal States. After the initial flooding, people struggled to obtain basic needs, including food, drinking water, safe shelter, clothing, medicines, personal hygiene items, and so forth, all as concern mounted about the safety of children and other relatives, friends, and neighbors. In this and similar cases, the destruction from the initial flooding led to mass displacement of families and communities; many people had to relocate far from New Orleans and other badly affected areas, while also needing to gain financial assistance, reinstate work to generate income, and obtain stable housing. People could not assimilate one stressor before another appeared.

Nevertheless, mass traumas can create an immediate sense of commonality—many people are “in the same boat,” thus removing much of the isolation that can occur with other types of trauma. People can acknowledge their difficulties and receive support, even from strangers. It is easier to ask for help because blame is often externalized; large-scale disasters are often referred to as “acts of God” or, in cases of terrorism and other intentional events, as acts of “evil.” Even so, survivors of mass trauma often encounter an initial rally of support followed by quickly diminishing services and dwindling care. When the disaster fades from the headlines, public attention and concern are likely to decrease, leaving survivors struggling to reestablish or reinvent their lives without much outside acknowledgment.

The experience of mass trauma can lead to the development of psychological symptoms and substance use at either a subclinical or a diagnostic level (refer to Part 3 of this TIP, available online, for more information highlighting the relationship between trauma and behavioral health problems). Likewise, one of the greatest risks for traumatic stress reactions after a mass tragedy is the presence of preexisting mental and co-occurring disorders, and individuals who are in early recovery from substance use disorders are at greater risk for such reactions as well. Nonetheless, people are amazingly resilient, and most will not develop long-term mental or substance use disorders after an event; in fact, most trauma-related symptoms will resolve in a matter of months (Keane & Piwawarczyk, 2006).

Interpersonal Traumas

Interpersonal traumas are events that occur (and typically continue to reoccur) between people who often know each other, such as spouses or parents and their children. Examples include physical and sexual abuse, sexual assault, domestic violence, and elder abuse.

Intimate partner violence

Intimate partner violence (IPV), often referred to as domestic violence, is a pattern of actual or threatened physical, sexual, and/or emotional abuse. It differs from simple assault in that multiple episodes often occur and the perpetrator is an intimate partner of the victim. Trauma associated with IPV is normally ongoing. Incidents of this form of violence are rarely isolated, and the client may still be in contact with and encountering abuse from the perpetrator while engaged in treatment.

Intimate partners include current and former spouses, boyfriends, and girlfriends. The majority of all nonfatal acts of violence and intimate partner homicides are committed against women; IPV accounts for over 20 percent of nonfatal violence against women but only 3.6 percent of that committed against men (Catalano, 2012). Children are the hidden casualties of IPV. They often witness the assaults or threats directly, within earshot, or by being exposed to the aftermath of the violence (e.g., seeing bruises and destruction of property, hearing the pleas for it to stop or the promises that it will never happen again).

Substance abuse, particularly involving alcohol, is frequently associated with IPV. It is the presence of alcohol-related problems in either partner, rather than the level of alcohol consumption itself, that is the important factor. Drinking may or may not be the cause of the violence; that said, couples with alcohol-related disorders could have more tension and disagreement within the relationship in general, which leads to aggression and violence. The consumption of alcohol during a dispute is likely to decrease inhibitions and increase impulsivity, thus creating an opportunity for an argument to escalate into a physical altercation. More information on domestic violence and its effects on partners and families, as well as its connection with substance use and trauma-related disorders, is available in TIP 25, Substance Abuse Treatment and Domestic Violence (CSAT, 1997b), and from the National Online Resource Center on Violence Against Women (<http://www.vawnet.org/>).

Developmental Traumas

Developmental traumas include specific events or experiences that occur within a given developmental stage and influence later development, adjustment, and physical and mental health. Often, these traumas are related to adverse childhood experiences (ACEs), but they can also result from tragedies that occur outside an expected developmental or life stage (e.g., a child dying before a parent, being diagnosed with a life-threatening illness as a young adult) or from events at any point in the life cycle that create significant loss and have life-altering consequences (e.g., the death of a significant other in the later years that leads to displacement of the surviving partner).

Adverse childhood experiences

Some people experience trauma at a young age through sexual, physical, or emotional abuse and neglect. The Adverse Childhood Experiences Study (Felitti et al., 1998) examined the effects of several categories of ACEs on adult health, including physical and emotional abuse; sexual abuse; a substance-dependent parent; an incarcerated, mentally ill, or suicidal household member; spousal abuse between parents; and divorce or separation that meant one parent was absent during childhood. The National Comorbidity Studies examined the prevalence of trauma and defined childhood adversities as parental death, parental divorce/separation, life-threatening illness, or extreme economic hardship in addition to the childhood experiences included in the Adverse Childhood Experiences Study (Green et al., 2010).

ACEs can negatively affect a person's wellbeing into adulthood. Whether or not these experiences occur simultaneously, are time-limited, or recur, they set the stage for increased vulnerability to physical, mental, and substance use disorders and enhance the risk for repeated trauma exposure across the life span. Childhood abuse is highly associated with major depression, suicidal thoughts, PTSD, and dissociative symptoms. So too, ACEs are associated with a greater risk of adult alcohol use. When a person experiences several adverse events in childhood, the risk of his or her heavy drinking, self-reported alcohol dependence, and marrying a person who is alcohol dependent is two to four times greater than that of a person with no ACEs (Dube, Anda, Felitti, Edwards, & Croft, 2002).

A detailed examination of the issues involved in providing substance abuse treatment to survivors of child abuse and neglect is the subject of TIP 36, Substance Abuse Treatment for Persons With Child Abuse and Neglect Issues (CSAT, 2000b).

Child Neglect

Child neglect occurs when a parent or caregiver does not give a child the care he or she needs according to his or her age, even though that adult can afford to give that care or is offered help to give that care. Neglect can mean not providing adequate nutrition, clothing, and/or shelter. It can mean that a parent or caregiver is not providing a child with medical or mental health treatment or is not giving prescribed medicines the child needs. Neglect can also mean neglecting the child's education. Keeping a child from school or from special education can be neglect. Neglect also includes exposing a child to dangerous environments (e.g., exposure

to domestic violence). It can mean poor supervision for a child, including putting the child in the care of someone incapable of caring for children. It can mean abandoning a child or expelling him or her from home. Lack of psychological care, including emotional support, attention, or love, is also considered neglect—and it is the most common form of abuse reported to child welfare authorities.

Source: dePanfilis, 2006.

Political Terror and War

Political terror and war are likely to have lasting consequences for survivors. In essence, anything that threatens the existence, beliefs, well-being, or livelihood of a community is likely to be experienced as traumatic by community members. Whether counselors are working with an immigrant or refugee enclave in the United States or in another country, they should be aware of local events, local history, and the possibility that clients have endured trauma. (For international information about the clinical, historical, and theoretical aspects of trauma and terrorism, see Danieli, Brom, & Sills, 2005.) Terrorism is a unique subtype of human-caused disasters. The overall goal of terrorist attacks is to maximize the uncertainty, anxiety, and fear of a large community, so the responses are often epidemic and affect large numbers of people who have had direct or indirect exposure to an event (Silver et al., 2004; Suvak, Maguen, Litz, Silver, & Holman, 2008). Terrorism has a variety of results not common to other disasters, such as reminders of the unpredictability of terrorist acts; increases in security measures for the general population; intensified suspicion about a particular population, ethnicity, or culture; and heightened awareness and/or arousal.

Refugees

According to the World Refugee Survey, there are an estimated 12 million refugees and asylum seekers, 21 million internally displaced people, and nearly 35 million uprooted people (U.S. Committee for Refugees and Immigrants, 2006). Many of these people have survived horrendous ordeals with profound and lasting effects for individuals and whole populations. In addition to witnessing deaths by execution, starvation, or beatings, many survivors have experienced horrific torture.

Torture and Captivity

Torture traumatizes by taking away an individual's personhood. To survive, victims have to give up their sense of self and will. They become the person the torturer designs or a nonperson, simply existing. Inevitably, the shame of the victim is enormous, because the focus of torture is to humiliate and degrade. As a result, victims often seek to hide their trauma and significant parts of their selfhood long after torture has ended and freedom has been obtained. According to Judith Herman, "the methods of establishing control over another person are based upon the systematic, repetitive infliction of psychological trauma. They are organized techniques of disempowerment and disconnection. Methods of psychological control are designed to instill terror and helplessness and to destroy the victim's sense of self in relation to others."

Source: Herman, 1997, p. 77.

Refugees are people who flee their homes because they have experienced or have a reasonable fear of experiencing persecution. They differ from immigrants who willingly leave their homes or homeland to seek better opportunities. Although immigrants may experience trauma before migrating to or after reaching their new destination, refugees will often have greater exposure to trauma before migration. Refugees typically come from wartorn countries and may have been persecuted or tortured. Consequently, greater exposure to trauma, such as torture, before migrating often leads to more adjustment-related difficulties and psychological symptoms after relocation (Steel et al., 2009).

Refugees typically face substantial difficulties in assimilating into new countries and cultures. Moreover, the environment can create a new set of challenges that may include additional exposure to trauma and social isolation (Miller et al., 2002). These as well as additional factors influence adjustment, the development of mental illness (including PTSD), and the occurrence of substance use disorders. Additional factors that influence outcomes after relocation include receptivity of the local community, along with opportunities for social support and culturally responsive services.

Among refugee populations in the United States, little research is available on rates of mental illness and co-occurring substance use disorders and traumatic stress among refugee populations. Substance use patterns vary based on cultural factors as well as assimilation, yet research suggests that trauma increases the risk for substance use among refugees after war-related experiences (Kozarić-Kovačić, Ljubin, & Grappe, 2000). Therefore, providers should expect to see trauma-related disorders among refugees who are seeking treatment for a substance use disorder and greater prevalence of substance use disorders among refugees who seek behavioral health services.

Vietnamese Refugees

“Wars always have consequences, both immediate and remote, and the consequences are often tragic. One tragic circumstance often caused by war is the forceful, disorganized, and uncontrollable mass movement of both civilians and soldiers trying to escape the horrors of the wars or of an oppressive regime...

“Vietnamese communists, by taking power in the North in 1954 and then in the South in 1975, caused two major upheavals in the Land of the Small Dragon, as Vietnam was once called. The first Vietnam War led to the 1954 exodus during which 1 million people fled from the North to the South. The second Vietnam War resulted in the dispersion, from 1975-1992, of approximately 2 million Vietnamese all over the world. These significant, unplanned, and uncoordinated mass movements around the world not only dislocated millions of people, but also caused thousands upon thousands of deaths at sea...

“The second and third wave of refugees from 1976 onward went through a more difficult time. They had to buy their way out and to hide from soldiers and the police who hunted them down. After catching them, the police either asked for bribes or threw the escapees into jails. Those who evaded police still had to face engine failures, sea storms, pirates... They then had to survive overcrowded boats for days or weeks, during which food and water could not be replenished and living conditions were terrible... Many people died from exhaustion, dehydration, and hunger. Others suffered at the hands of terrifying pirates... After the sea ordeal came the overcrowded camps where living conditions were most often substandard and where security was painfully lacking...

“In the United States, within less than 3 decades, the Vietnamese population grew from a minority of perhaps 1,000 persons to the second largest refugee group behind Cubans.”

Source: Vo, 2006, pp. 1–4.

System-Oriented Traumas: Retraumatization

Retraumatization occurs when clients experience something that makes them feel as though they are undergoing another trauma. Unfortunately, treatment settings and clinicians can create retraumatizing experiences, often without being aware of it, and sometimes clients themselves are not consciously aware that a clinical situation has actually triggered a traumatic stress reaction. Agencies that anticipate the risk for retraumatization and actively work on adjusting program policies and procedures to remain sensitive to the histories and needs of individuals who have undergone past trauma are likely to have more success in providing care, retaining clients, and achieving positive outcomes.

Staff and agency issues that can cause retraumatization include:

- Being unaware that the client’s traumatic history significantly affects his or her life.
- Failing to screen for trauma history prior to treatment planning.
- Challenging or discounting reports of abuse or other traumatic events.
- Using isolation or physical restraints.
- Using experiential exercises that humiliate the individual.
- Endorsing a confrontational approach in counseling.
- Allowing the abusive behavior of one client toward another to continue without intervention.
- Labeling behavior/feelings as pathological.
- Failing to provide adequate security and safety within the program.
- Limiting participation of the client in treatment decisions and planning processes.
- Minimizing, discrediting, or ignoring client responses.
- Disrupting counselor–client relationships by changing counselors’ schedules and assignments.
- Obtaining urine specimens in a nonprivate setting.
- Having clients undress in the presence of others.
- Inconsistently enforcing rules and allowing chaos in the treatment environment.
- Imposing agency policies or rules without exceptions or an opportunity for clients to question them.
- Enforcing new restrictions within the program without staff–client communication.
- Limiting access to services for ethnically diverse populations.
- Accepting agency dysfunction, including lack of consistent, competent leadership.

Advice to Counselors: Addressing Retraumatization

- Anticipate and be sensitive to the needs of clients who have experienced trauma regarding program policies and procedures in the treatment setting that might trigger memories of trauma, such as lack of privacy, feeling pushed to take psychotropic medications, perceiving that they have limited choices within the program or in the selection of the program, and so forth.
- Attend to clients' experiences. Ignoring clients' behavioral and emotional reactions to having their traumatic memories triggered is more likely to increase these responses than decrease them.
- Develop an individual coping plan in anticipation of triggers that the individual is likely to experience in treatment based on his or her history.
- Rehearse routinely the coping strategies highlighted in the coping plan. If the client does not practice strategies prior to being triggered, the likelihood of being able to use them effectively upon triggering is lessened. For example, it is far easier to practice grounding exercises in the absence of severe fear than to wait for that moment when the client is reexperiencing an aspect of a traumatic event. (For more information on grounding exercises, refer to *Seeking Safety: A Treatment Manual for PTSD and Substance Abuse*; Najavits, 2002a, pp. 125–131.)
- Recognize that clinical and programmatic efforts to control or contain behavior in treatment can cause traumatic stress reactions, particularly for trauma survivors for whom being trapped was part of the trauma experience.
- Listen for the specific trigger that seems to be driving the client's reaction. It will typically help both the counselor and client understand the behavior and normalize the traumatic stress reactions.
- Make sure that staff and other clients do not shame the trauma survivor for his or her behavior, such as through teasing or joking about the situation.
- Respond with consistency. The client should not get conflicting information or responses from different staff members; this includes information and responses given by administrators.

Characteristics of Trauma

The following section highlights several selected characteristics of traumatic experiences that influence the effects of traumatic stress. Objective characteristics are those elements of a traumatic event that are tangible or factual; subjective characteristics include internal processes, such as perceptions of traumatic experiences and meanings assigned to them.

Objective Characteristics

Was it a single, repeated, or sustained trauma?

Trauma can involve a single event, numerous or repeated events, or sustained/chronic experiences. A single trauma is limited to a single point in time. A rape, an automobile accident, the sudden death of a loved one—all are examples of a single trauma. Some people who experience a single trauma recover without any specific intervention. But for others—especially those with histories of previous trauma or mental or substance use disorders, or those for whom the trauma experience is particularly horrific or overwhelming—a single trauma can result in traumatic stress symptoms and trauma- and stress-related disorders. Single traumas do not necessarily have a lesser psychological impact than repeated traumas.

After the terrorist attacks on September 11, 2001—a significant single trauma—many Manhattan residents experienced intrusive memories and sleep disruption whether they were at the site of the attacks or watched television coverage of it (Ford & Fournier, 2007; Galea et al., 2002).

A series of traumas happening to the same person over time is known as repeated trauma. This can include repeated sexual or physical assaults, exposure to frequent injuries of others, or seemingly unrelated traumas. Military personnel, journalists covering stories of mass tragedies or prolonged conflicts, and first responders who handle hundreds of cases each year typify repeated trauma survivors. Repetitive exposure to traumas can have a cumulative effect over one's lifetime. A person who was assaulted during adolescence, diagnosed with a life-threatening illness in his or her thirties, and involved in a serious car accident later in life has experienced repeated trauma.

Some repeated traumas are sustained or chronic. Sustained trauma experiences tend to wear down resilience and the ability to adapt. Some examples include children who endure ongoing sexual abuse, physical neglect, or emotional abuse; people who are in violent relationships; and people who live in chronic poverty. Individuals in chronically stressful, traumatizing environments are particularly susceptible to traumatic stress reactions, substance use, and mental disorders.

Bidirectional relationships exist between trauma and substance use as well as trauma and mental illness. For example, abuse of alcohol and drugs increases the risk of a traumatic experience and creates greater vulnerability to the effects of trauma; substance

abuse reduces a person's ability to take corrective and remedial actions that might reduce the impact of the trauma. Likewise, traumatic stress leads to a greater likelihood of substance abuse that, in turn, increases the risk for additional exposure to trauma. Paralleling this bidirectional relationship, mental illness increases vulnerability to the effects of trauma and raises the risk for substance use disorders and for encountering additional traumatic events. So too, early exposure to ACEs is associated with traumatic stress reactions and subsequent exposure to trauma in adult years.

People who have encountered multiple and longer doses of trauma are at the greatest risk for developing traumatic stress. For example, military reservists and other military service members who have had multiple long tours of duty are at greater risk for traumatic stress reactions (see the planned TIP, Reintegration-Related Behavioral Health Issues in Veterans and Military Families; SAMHSA, planned f). In addition, people are more likely to encounter greater impairment and distress from trauma if that trauma occurs with significant intensity and continues sporadically or unceasingly for extended periods.

Case Illustration: Yourself

Think of a time that was particularly stressful (but not traumatic) in your life. Revisit this period as an observer watching the events unfold and then ask yourself, "What made this time particularly stressful?" It is likely that a part of your answer will include the difficulty of managing one situation before another circumstance came along demanding your time. Stressful times denote being bombarded with many things at one time, perceived or actual, without sufficient time or ability to address them emotionally, cognitively, spiritually, and/or physically. The same goes for trauma—rapid exposure to numerous traumas one after another lessens one's ability to process the event before the next onslaught. This creates a cumulative effect, making it more difficult to heal from any one trauma.

Was there enough time to process the experience?

A particularly severe pattern of ongoing trauma, sometimes referred to as "cascading trauma," occurs when multiple traumas happen in a pattern that does not allow an individual to heal from one traumatic event before another occurs. Take, for example, California residents—they repeatedly face consecutive and/or simultaneous natural disasters including fires, landslides, floods, droughts, and earthquakes. In other cases, there is ample time to process an event, but processing is limited because people don't have supportive relationships or environments that model preventive practices. This can lead to greater vulnerability to traumas that occur later in life.

How many losses has the trauma caused?

Trauma itself can create significant distress, but often, the losses associated with a trauma have more far-reaching effects. For instance, a child may be forced to assume adult responsibilities, such as serving as a confidant for a parent who is sexually abusing him or her, and lose the opportunity of a childhood free from adult worries. In another scenario, a couple may initially feel grateful to have escaped a house fire, but they may nevertheless face significant community and financial losses months afterward. In evaluating the impact of trauma, it is helpful to access and discuss the losses associated with the initial trauma. The number of losses greatly influences an individual's ability to bounce back from the tragedy.

In the case illustration on the next page, Rasheed's losses cause him to disconnect from his wife, who loves and supports him. Successful confrontation of losses can be difficult if the losses compound each other, as with Rasheed's loss of his friend, his disability, his employment struggles, and the threats to his marriage and liberty. People can cite a specific event as precipitating their trauma, or, in other cases, the specific trauma can symbolize a series of disabling events in which the person felt his or her life was threatened or in which he or she felt emotionally overwhelmed, psychologically disorganized, or significantly disconnected from his or her surroundings. It will be important for Rasheed to understand how his losses played a part in his abuse of prescription medications to cope with symptoms associated with traumatic stress and loss, (e.g., guilt, depression, fear). If not addressed, his trauma could increase his risk for relapse.

Case Illustration: Rasheed

Rasheed was referred to an employee assistance program by his employer. He considered quitting his job, but his wife insisted he talk to a counselor. He is a 41-year-old auto mechanic who, 4 years ago, caused a head-on collision while attempting to pass another vehicle. A close friend, riding in the passenger's seat, was killed, and two young people in the other vehicle were seriously injured and permanently disabled. Rasheed survived with a significant back injury and has only been able to work sporadically. He was convicted of negligent homicide and placed on probation because of his physical disability. He is on probation for another 4 years, and if he is convicted of another felony during that time, he will have to serve prison time for his prior offense.

While still in the hospital, Rasheed complained of feeling unreal, numb, and disinterested in the care he received. He did not remember the crash but remembers waking up in the hospital 2 days later. He had difficulty sleeping in the hospital and was aware of feelings of impending doom, although he was unaware of the legal charges he would later face. He was diagnosed with ASD.

He was discharged from the hospital with a variety of medications, including pain pills and a sleep aid. He rapidly became dependent on these medications, feeling he could not face the day without the pain medication and being unable to sleep without sleep medicine in larger doses than had been prescribed. Within 3 months of the accident, he was “doctor shopping” for pain pills and even had a friend obtain a prescription for the sleeping medication from that friend’s doctor. In the 4 intervening years, Rasheed’s drug use escalated, and his blunted emotions and detachment from friends became more profound. He became adept at obtaining pain pills from a variety of sources, most of them illegal. He fears that if he seeks treatment for the drug problem, he will have to admit to felony offenses and will probably be imprisoned. He also does not believe he can manage his life without the pain pills.

In the past 2 years, he has had recurring dreams of driving a car on the wrong side of the road and into the headlights of an oncoming vehicle. In the dream, he cannot control the car and wakes up just before the vehicles crash. At unusual times—for instance, when he is just awakening in the morning, taking a shower, or walking alone—he will feel profound guilt over the death of his friend in the accident. He becomes very anxious when driving in traffic or when he feels he is driving faster than he should. His marriage of 18 years has been marked by increasing emotional distance, and his wife has talked about separating if he does not do something about his problem. He has been unable to work consistently because of back pain and depression. He was laid off from one job because he could not concentrate and was making too many mistakes.

The counselor in the employee assistance program elicited information on Rasheed’s drug use, although she suspected Rasheed was minimizing its extent and effects. Knowledgeable about psychological trauma, the counselor helped Rasheed feel safe enough to talk about the accident and how it had affected his life. She was struck by how little Rasheed connected his present difficulties to the accident and its aftermath. The counselor later commented that Rasheed talked about the accident as if it had happened to someone else. Rasheed agreed to continue seeing the counselor for five additional visits, during which time a plan would be made for Rasheed to begin treatment for drug dependence and PTSD.

Was the trauma expected or unexpected?

When talking about a trauma, people sometimes say they didn’t see it coming. Being unprepared, unaware, and vulnerable often increases the risk of psychological injury, but these are common components of most traumas, given that most traumatic events do occur without warning (e.g., car crashes, terrorist attacks, sexual assaults). People with substance use disorders, mental illness, and/or cognitive disabilities may be especially vulnerable in that they may attend less or have competing concerns that diminish attention to what is going on around them, even in high-risk environments. However, most individuals attempt to gain some control over the tragedy by replaying the moments leading up to the event and processing how they could have anticipated it. Some people persevere on these thoughts for months or years after the event.

Sometimes, a trauma is anticipated but has unexpected or unanticipated consequences, as in the case of Hurricane Katrina. Learning about what is likely to happen can reduce traumatization. For instance, training military personnel in advance of going to combat overseas prepares them to handle traumas and can reduce the impact of trauma.

Were the trauma’s effects on the person’s life isolated or pervasive?

When a trauma is isolated from the larger context of life, a person’s response to it is more likely to be contained and limited. For instance, military personnel in combat situations can be significantly traumatized by what they experience. On return to civilian life or non-combat service, some are able to isolate the traumatic experience so that it does not invade ordinary, day-to-day living. This does not mean that the combat experience was not disturbing or that it will not resurface if the individual encounters an experience that triggers memories of the trauma; it just means that the person can more easily leave the trauma in the past and attend to the present.

Conversely, people who remain in the vicinity of the trauma may encounter greater challenges in recovery. The traumatic event intertwines with various aspects of the person’s daily activities and interactions, thus increasing the possibility of being triggered by surrounding cues and experiencing subsequent psychological distress. However, another way to view this potential dilemma for the client is to reframe it as an opportunity—the repetitive exposure to trauma-related cues may provide vital guidance as to when and which treatment and coping techniques to use in the delivery of trauma-informed and trauma-specific behavioral health services.

Who was responsible for the trauma and was the act intentional?

If the severity of a trauma is judged solely by whether the act was intentional or not, events that reflect an intention to harm would be a primary indicator in predicting subsequent difficulties among individuals exposed to this form of trauma. For most survivors, there is an initial disbelief that someone would conceivably intend to harm others, followed by considerable emotional and, at times, behavioral investment in somehow making things right again or in making sense of a senseless, malicious act. For instance, in the wake of the World Trade Center attacks in New York City, people responded via renewed patriotism, impromptu candlelight vigils, attacks on people of Arab and Muslim descent, and unprecedented donations and willingness to wait in long lines to donate blood to the Red Cross. Each example is a response that in some way attempts to right the perceived wrong or attach new meaning to the event and subsequent consequences.

When terrible things happen, it is human nature to assign blame. Trauma survivors can become heavily invested in assigning blame or finding out who was at fault, regardless of the type of trauma. Often, this occurs as an attempt to make sense of, give meaning to, and reestablish a sense of predictability, control, and safety after an irrational or random act. It is far easier to accept that someone, including oneself, is at fault or could have done something different than it is to accept the fact that one was simply in the wrong place at the wrong time.

For some trauma survivors, needing to find out why a trauma occurred or who is at fault can become a significant block to growth when the individual would be better served by asking, “What do I need to do to heal?” Behavioral health professionals can help clients translate what they have learned about responsibility in recovery to other aspects of their lives. For instance, someone in treatment for co-occurring disorders who has internalized that becoming depressed or addicted was not his or her fault, but that recovery is a personal responsibility, can then apply the same principle to the experience of childhood abuse and thereby overcome negative judgments of self (e.g., thinking oneself to be a bad person who deserves abuse). The individual can then begin to reassign responsibility by attaching the blame to the perpetrator(s) while at the same time assuming responsibility for recovery.

Was the trauma experienced directly or indirectly?

Trauma that happens to someone directly seems to be more damaging than witnessing trauma that befalls others. For example, it is usually more traumatic to be robbed at gunpoint than to witness someone else being robbed or hearing someone tell a story about being robbed. Yet, sometimes, experiencing another’s pain can be equally traumatic. For instance, parents often internalize the pain and suffering of their children when the children are undergoing traumatic circumstances (e.g., treatments for childhood cancer).

There are two ways to experience the trauma of others. An individual may witness the event, such as seeing someone killed or seriously injured in a car accident, or may learn of an event that happened to someone, such as a violent personal assault, suicide, serious accident, injury, or sudden or unexpected death. For many people, the impact of the trauma will depend on a host of variables, including their proximity to the event as eyewitnesses, the witnesses’ response in the situation, their relationship to the victims, the degree of helplessness surrounding the experience, their exposure to subsequent consequences, and so on.

The effects of traumas such as genocide and internment in concentration camps can be felt across generations—stories, coping behaviors, and stress reactions can be passed across generational lines far removed from the actual events or firsthand accounts. Known as historical trauma, this type of trauma can affect the functioning of families, communities, and cultures for multiple generations.

Case Illustration: Frank

Frank entered substance abuse treatment with diagnoses of co-occurring PTSD and substance use disorder. While on a whitewater kayak trip with his wife, her kayak became pinned on a rock, and Frank could only watch helplessly as she drowned. His drinking had increased markedly after the accident. He acknowledged a vicious cycle of sleep disturbance with intrusive nightmares followed by vivid memories and feelings of terror and helplessness after he awoke. He drank heavily at night to quiet the nightmares and memories, but heavy alcohol consumption perpetuated his trouble sleeping. He withdrew from contact with many of his old “couple friends” and his wife’s family, with whom he had been close. At treatment entry, he described his life as “going to work and coming home.” The trauma occurred 3 years before he sought treatment, but Frank continued to feel numb and disconnected from the world. His only emotion was anger, which he tried to keep in check. Integrated treatment for PTSD and substance abuse helped him sleep and taught him coping skills to use when the memories arose; it fostered his engagement and retention in long-term care for both disorders.

What happened since the trauma?

In reviewing traumatic events, it is important to assess the degree of disruption after the initial trauma has passed, such as the loss of employment, assets, community events, behavioral health services, local stores, and recreational areas. There is typically an initial rally of services and support following a trauma, particularly if it is on a mass scale. However, the reality of the trauma's effects and their disruptiveness may have a more lasting impact. The deterioration of normalcy, including the disruption of day-to-day activities and the damage of structures that house these routines, will likely erode the common threads that provide a sense of safety in individual lives and communities. Hence, the degree of disruption in resuming normal daily activities is a significant risk factor for substance use disorders, subclinical psychological symptoms, and mental disorders. For example, adults displaced from their homes because of Hurricanes Katrina or Rita had significantly higher rates of past-month cigarette use, illicit drug use, and binge drinking than those who were not displaced (Office of Applied Studies, 2008).

Subjective Characteristics

Psychological meaning of trauma

An important clinical issue in understanding the impact of trauma is the meaning that the survivor has attached to the traumatic experience. Survivors' unique cognitive interpretations of an event—that is, their beliefs and assumptions—contribute to how they process, react to, cope with, and recover from the trauma. Does the event represent retribution for past deeds committed by the individual or his or her family? How does the individual attach meaning to his or her survival? Does he or she believe that it is a sign of a greater purpose not yet revealed? People who attempt to share their interpretation and meaning of the event can feel misunderstood and sometimes alienated (Paulson & Krippner, 2007; Schein, Spitz, Burlingame, & Muskin, 2006).

It is important to remember that what happened is not nearly as important as what the trauma means to the individual.

People interpret traumatic events in vastly different ways, and many variables shape how an individual assigns meaning to the experience (framing the meaning through culture, family beliefs, prior life experiences and learning, personality and other psychological features, etc.). Even in an event that happens in a household, each family member may interpret the experience differently. Likewise, the same type of event can occur at two different times in a person's life, but his or her interpretation of the events may differ considerably because of developmental differences acquired between events, current cognitive and emotional processing skills, availability of and access to environmental resources, and so forth.

Disruption of core assumptions and beliefs

Trauma often engenders a crisis of faith (Frankl, 1992) that leads clients to question basic assumptions about life. Were the individual's core or life-organizing assumptions (e.g., about safety, perception of others, fairness, purpose of life, future dreams) challenged or disrupted during or after the traumatic event? (See the seminal work, *Shattered Assumptions*, by Janoff-Bulman, 1992.) For example, some trauma survivors see themselves as irreparably wounded or beyond the possibility of healing. The following case illustration (Sonja) explores not only the importance of meaning, but also the role that trauma plays in altering an individual's core assumptions—the very assumptions that provide meaning and a means to organize our lives and our interactions with the world and others.

Resilience: Connection and Continuity

Research suggests that reestablishing ties to family, community, culture, and spiritual systems is not only vital to the individual, but it also influences the impact of the trauma upon future generations. For example, Baker and Gippenreiter (1998) studied the descendants of survivors of Joseph Stalin's purge. They found that families who were able to maintain a sense of connection and continuity with grandparents directly affected by the purge experienced fewer negative effects than those who were emotionally or physically severed from their grandparents. Whether the grandparents survived was less important than the connection the grandchildren felt to their pasts.

Cultural meaning of trauma

Counselors should strive to appreciate the cultural meaning of a trauma. How do cultural interpretations, cultural support, and cultural responses affect the experience of trauma? It is critical that counselors do not presume to understand the meaning of a traumatic experience without considering the client's cultural context. Culture strongly influences the perceptions of trauma. For instance, a trauma involving shame can be more profound for a person from an Asian culture than for someone from a European culture. Likewise, an Alaska Native individual or community, depending upon their Tribal ancestry, may believe that the traumatic experience serves as a form of retribution. Similarly, the sudden death of a family member or loved one can be less traumatic in a

culture that has a strong belief in a positive afterlife. It is important for counselors to recognize that their perceptions of a specific trauma could be very different from their clients' perceptions. Be careful not to judge a client's beliefs in light of your own value system. For more information on culture and how to achieve cultural competence in providing behavioral health services, see SAMHSA's planned TIP, Improving Cultural Competence (SAMHSA, planned c).

Individual & Sociocultural Features

A wide variety of social, demographic, environmental, and psychological factors influence a person's experience of trauma, the severity of traumatic stress reactions following the event, and his or her resilience in dealing with the short- and long-term environmental, physical, sociocultural, and emotional consequences. This section addresses a few known factors that influence the risk of trauma along with the development of subclinical and diagnostic traumatic stress symptoms, such as mood and anxiety symptoms and disorders. It is not meant to be an exhaustive exploration of these factors, but rather, a brief presentation to make counselors and other behavioral health professionals aware that various factors influence risk for and protection against traumatic stress and subsequent reactions. (For a broader perspective on such factors, refer to Part 1, Chapter 1.)

Individual Factors

Several factors influence one's ability to deal with trauma effectively and increase one's risk for traumatic stress reactions. Individual factors pertain to the individual's genetic, biological, and psychological makeup and history as they influence the person's experience and interpretation of, as well as his or her reactions to, trauma. However, many factors influence individual responses to trauma; it is not just individual characteristics. Failing to recognize that multiple factors aside from individual attributes and history influence experiences during and after trauma can lead to blaming the victim for having traumatic stress.

Case Illustration: Sonja

Sonja began to talk about how her life was different after being physically assaulted and robbed in a parking lot at a local strip mall a year ago. She recounts that even though there were people in the parking lot, no one came to her aid until the assailant ran off with her purse. She sustained a cheekbone fracture and developed visual difficulties due to the inflammation from the fracture. She recently sought treatment for depressive symptoms and reported that she had lost interest in activities that typically gave her joy. She reported isolating herself from others and said that her perception of others had changed dramatically since the attack.

Sonja had received a diagnosis of major depression with psychotic features 10 years earlier and received group therapy at a local community mental health center for 3 years until her depression went into remission. She recently became afraid that her depression was becoming more pronounced, and she wanted to prevent another severe depressive episode as well as the use of psychotropic medications, which she felt made her lethargic. Thus, she sought out behavioral health counseling.

As the sessions progressed, and after a psychological evaluation, it was clear that Sonja had some depressive symptoms, but they were subclinical. She denied suicidal thoughts or intent, and her thought process was organized with no evidence of hallucinations or delusions. She described her isolation as a reluctance to shop at area stores. On one hand, Sonja was self-compassionate about her reasons for avoidance, but on the other hand, she was concerned that the traumatic event had altered how she saw life and others. "I don't see people as very caring or kind, like I used to prior to the event. I don't trust them, and I feel people are too self-absorbed. I don't feel safe, and this bothers me. I worry that I'm becoming paranoid again. I guess I know better, but I just want to have the freedom to do what I want and go where I want."

Two months after Sonja initiated counseling, she came to the office exclaiming that things can indeed change. "You won't believe it. I had to go to the grocery store, so I forced myself to go the shopping center that had a grocery store attached to a strip mall. I was walking by a coffee shop, quickly browsing the items in the front window, when a man comes out of the shop talking at me. He says, 'You look like you need a cup of coffee.' What he said didn't register immediately. I looked at him blankly, and he said it again. 'You look like you need a cup of coffee. I'm the owner of the shop, and I noticed you looking in the window, and we have plenty of brewed coffee left before we close the shop. Come on in, it's on the house.' So I did! From that moment on, I began to see people differently. He set it right for me—I feel as if I have myself back again, as if the assault was a sign that I shouldn't trust people, and now I see that there is some goodness in the world. As small as this kindness was, it gave me the hope that I had lost."

For Sonja, the assault changed her assumptions about safety and her view of others. She also attached meaning to the event. She believed that the event was a sign that she shouldn't trust people and that people are uncaring. Yet these beliefs bothered her and contradicted how she saw herself in the world, and she was afraid that her depressive symptoms were returning.

For an inexperienced professional, her presentation may have ignited suspicions that she was beginning to present with psychotic features. However, it is common for trauma survivors to experience changes in core assumptions immediately after the event and to attach meaning to the trauma. Often, a key ingredient in the recovery process is first identifying the meaning of the event and the beliefs that changed following the traumatic experience. So when you hear a client say “I will never see life the same,” this expression should trigger further exploration into how life is different, what meaning has been assigned to the trauma, and how the individual has changed his or her perception of self, others, and the future.

Sometimes, reworking the altered beliefs and assumptions occurs with no formal intervention, as with Sonja. In her situation, a random stranger provided a moment that challenged an assumption generated from the trauma. For others, counseling may be helpful in identifying how beliefs and thoughts about self, others, and the world have changed since the event and how to rework them to move beyond the trauma. It is important to understand that the meaning that an individual attaches to the event(s) can either undermine the healing process (e.g., believing that you should not have survived, feeling shame about the trauma, continuing to engage in high-risk activities) or pave the road to recovery (e.g., volunteering to protect victim rights after being sexually assaulted). The following questions can help behavioral health staff members introduce topics surrounding assumptions, beliefs, interpretations, and meanings related to trauma:

- In what ways has your life been different since the trauma?
- How do you understand your survival? (This is an important question for clients who have been exposed to ACEs or cumulative trauma and those who survived a tragedy when others did not.)
- Do you believe that there are reasons that this event happened to you? What are they?
- What meaning does this experience have for you?
- Do you feel that you are the same person as before the trauma? In what ways are you the same? In what ways do you feel different?
- How did this experience change you as a person? Would you like to return to the person you once were? What would you need to do, or what would need to happen, for this to occur?
- Did the traumatic experience change you in a way that you don’t like? In what ways?
- How do you view others and your future differently since the trauma?
- What would you like to believe now about the experience?

History of prior psychological trauma

People with histories of prior psychological trauma appear to be the most susceptible to severe traumatic responses (Nishith, Mechanic, & Resick, 2000; Vogt, Bruce, Street, & Stafford, 2007), particularly if they have avoided addressing past traumas. Because minimization, dissociation, and avoidance are common defenses for many trauma survivors, prior traumas are not always consciously available, and when they are, memories can be distorted to avoid painful affects. Some survivors who have repressed their experiences deny a history of trauma or are unable to explain their strong reactions to present situations.

Remember that the effects of trauma are cumulative; therefore, a later trauma that outwardly appears less severe may have more impact upon an individual than a trauma that occurred years earlier. Conversely, individuals who have experienced earlier traumas may have developed effective coping strategies or report positive outcomes as they have learned to adjust to the consequences of the trauma(s). This outcome is often referred to as posttraumatic growth or psychological growth.

Clients in behavioral health treatment who have histories of trauma can respond negatively to or seem disinterested in treatment efforts. They may become uncomfortable in groups that emphasize personal sharing; likewise, an individual who experiences brief bouts of dissociation (a reaction of some trauma survivors) may be misunderstood by others in treatment and seen as uninterested. Providers need to attend to histories, adjust treatment to avoid retraumatization, and steer clear of labeling clients’ behavior as pathological.

History of resilience

Resilience—the ability to thrive despite negative life experiences and heal from traumatic events—is related to the internal strengths and environmental supports of an individual. Most individuals are resilient despite experiencing traumatic stress. The ability to thrive beyond the trauma is associated with individual factors as well as situational and contextual factors. There are not only one or two primary factors that make an individual resilient; many factors contribute to the development of resilience. There is little research to indicate that there are specific traits predictive of resilience; instead, it appears that more general characteristics influence resilience, including neurobiology (Feder, Charney, & Collins, 2011), flexibility in adapting to change, beliefs prior to trauma, sense of self-efficacy, and ability to experience positive emotions (Bonanno & Mancini, 2011).

History of mental disorders

The correlations among traumatic stress, substance use disorders, and co-occurring mental disorders are well known. According to the

Diagnostic and Statistical Manual of Mental Disorders, Fifth Edition (American Psychiatric Association, 2013a), traumatic stress reactions are linked to higher rates of mood, substance-related, anxiety, trauma, stress-related, and other mental disorders, each of which can precede, follow, or emerge concurrently with trauma itself. A co-occurring mental disorder is a significant determinant of whether an individual can successfully address and resolve trauma as it emerges from the past or occurs in the present. Koenen, Stellman, Stellman, and Sommer (2003) found that the risk of developing PTSD following combat trauma was higher for individuals with preexisting conduct disorder, panic disorder, generalized anxiety disorder, and/or major depression than for those without preexisting mental disorders. For additional information on comorbidity of trauma and other mental disorders, see TIP 42, Substance Abuse Treatment for Persons With Co-Occurring Disorders (CSAT, 2005c).

Sociodemographic Factors

Demographic variables are not good predictors of who will experience trauma and subsequent traumatic stress reactions. Gender, age, race and ethnicity, sexual orientation, marital status, occupation, income, and education can all have some influence, but not enough to determine who should or should not receive screening for trauma and traumatic stress symptoms. The following sections cover a few selected variables. (For more information, please refer to Part 3 of this TIP, the online literature review.)

Gender

In the United States, men are at greater risk than women for being exposed to stressful events. Despite the higher prevalence among men, lifetime PTSD occurs at about twice the rate among women as it does in men. Less is known about gender differences with subclinical traumatic stress reactions. There are also other gender differences, such as the types of trauma experienced by men and women. Women are more likely to experience physical and sexual assault, whereas men are most likely to experience combat and crime victimization and to witness killings and serious injuries (Breslau, 2002; Kimerling, Ouimette, & Weitlauf, 2007; Tolin & Foa, 2006). Women in military service are subject to the same risks as men and are also at a greater risk for military sexual trauma. Men's traumas often occur in public; women's are more likely to take place in private settings. Perpetrators of traumas against men are often strangers, but women are more likely to know the perpetrator.

Age

In general, the older one becomes, the higher the risk of trauma—but the increase is not dramatic. Age is not particularly important in predicting exposure to trauma, yet at no age is one immune to the risk. However, trauma that occurs in the earlier and midlife years appears to have greater impact on people for different reasons. For younger individuals, the trauma can affect developmental processes, attachment, emotional regulation, life assumptions, cognitive interpretations of later experiences, and so forth (for additional resources, visit the National Child Traumatic Stress Network; <http://www.nctsn.org/>). For adults in midlife, trauma may have a greater impact due to the enhanced stress or burden of care that often characterizes this stage of life—caring for their children and their parents at the same time. Older adults are as likely as younger adults to recover quickly from trauma, yet they may have greater vulnerabilities, including their ability to survive without injury and their ability to address the current trauma without psychological interference from earlier stressful or traumatic events. Older people are naturally more likely to have had a history of trauma because they have lived longer, thus creating greater vulnerability to the effects of cumulative trauma.

Race, ethnicity, and culture

The potential for trauma exists in all major racial and ethnic groups in American society, yet few studies analyze the relationship of race and ethnicity to trauma exposure and/or traumatic stress reactions. Some studies show that certain racial and ethnic groups are at greater risk for specific traumas. For example, African Americans experienced higher rates of overall violence, aggravated assault, and robbery than Whites but were as likely to be victims of rape or sexual assault (Catalano, 2004). Literature reflects that diverse ethnic, racial, and cultural groups are more likely to experience adverse effects from various traumas and to meet criteria for posttraumatic stress (Bell, 2011).

Resilience: Cultural, Racial, and Ethnic Characteristics

The following list highlights characteristics that often nurture resilience among individuals from diverse cultural, racial, and ethnic groups:

- Strong kinship bonds

- Respect for elders and the importance of extended family
- Spirituality and religious practices (e.g., shrine visitations or the use of traditional healers)
- Value in friendships and warm personal relationships
- Expression of humor and creativity
- Instilling a sense of history, heritage, and historical traditions
- Community orientation, activities, and socialization
- Strong work ethic
- Philosophies and beliefs about life, suffering, and perseverance

“Fortune owes its existence to misfortune, and misfortune is hidden in fortune.” –Lao-Tzu teaching, Taoism (Wong & Wong, 2006)

Sexual orientation and gender identity

Lesbian, gay, bisexual, and transgender individuals are likely to experience various forms of trauma associated with their sexual orientation, including harsh consequences from families and faith traditions, higher risk of assault from casual sexual partners, hate crimes, lack of legal protection, and laws of exclusion (Brown, 2008). Gay and bisexual men as well as transgender people are more likely to experience victimization than lesbians and bisexual women. Dillon (2001) reported a trauma exposure rate of 94 percent among lesbian, gay, and bisexual individuals; more than 40 percent of respondents experienced harassment due to their sexual orientation. Heterosexual orientation is also a risk for women, as women in relationships with men are at a greater risk of being physically and sexually abused.

People who are homeless

Homelessness is typically defined as the lack of an adequate or regular dwelling, or having a nighttime dwelling that is a publicly or privately supervised institution or a place not intended for use as a dwelling (e.g., a bus station). The U.S. Department of Housing and Urban Development (HUD) estimates that between 660,000 and 730,000 individuals were homeless on any given night in 2005 (HUD, 2007). Two thirds were unaccompanied persons; the other third were people in families. Adults who are homeless and unmarried are more likely to be male than female. About 40 percent of men who are homeless are veterans (National Coalition for the Homeless, 2002); this percentage has grown, including the number of veterans with dependent children (Kuhn & Nakashima, 2011).

Rates of trauma symptoms are high among people who are homeless (76 to 100 percent of women and 67 percent of men; Christensen et al., 2005; Jainchill, Hawke, & Yagelka, 2000), and the diagnosis of PTSD is among the most prevalent non-substance use Axis I disorders (Lester et al., 2007; McNamara, Schumacher, Milby, Wallace, & Usdan, 2001). People who are homeless report high levels of trauma (especially physical and sexual abuse in childhood or as adults) preceding their homeless status; assault, rape, and other traumas frequently happen while they are homeless. Research suggests that many women are homeless because they are fleeing domestic violence (National Coalition for the Homeless, 2002). Other studies suggest that women who are homeless are more likely to have histories of childhood physical and sexual abuse and to have experienced sexual assault as adults. A history of physical and/or sexual abuse is even more common among women who are homeless and have a serious mental illness.

Youth who are homeless, especially those who live without a parent, are likely to have experienced physical and/or sexual abuse. Between 21 and 42 percent of youth runaways report having been sexually abused before leaving their homes; for young women, rates range from 32 to 63 percent (Administration on Children, Youth and Families, 2002). Additionally, data reflect elevated rates of substance abuse for youth who are homeless and have histories of abuse.

More than half of people who are homeless have a lifetime prevalence of mental illness and substance use disorders. Those who are homeless have higher rates of substance abuse (84 percent of men and 58 percent of women), and substance use disorders, including alcohol and drug abuse/dependence, increase with longer lengths of homelessness (North, Eyrich, Pollio, & Spitznagel, 2004).

For more information on providing trauma-informed behavioral health services to clients who are homeless, and for further discussion of the incidence of trauma in this population, see TIP 55-R, Behavioral Health Services for People Who Are Homeless (SAMHSA, 2013b).

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